



STRATEGIC INFORMATION COMPONENT MONTHLY REPORT

REPORTING MONTH – AUGUST 2026

REPORTING PERIOD – JULY 2026

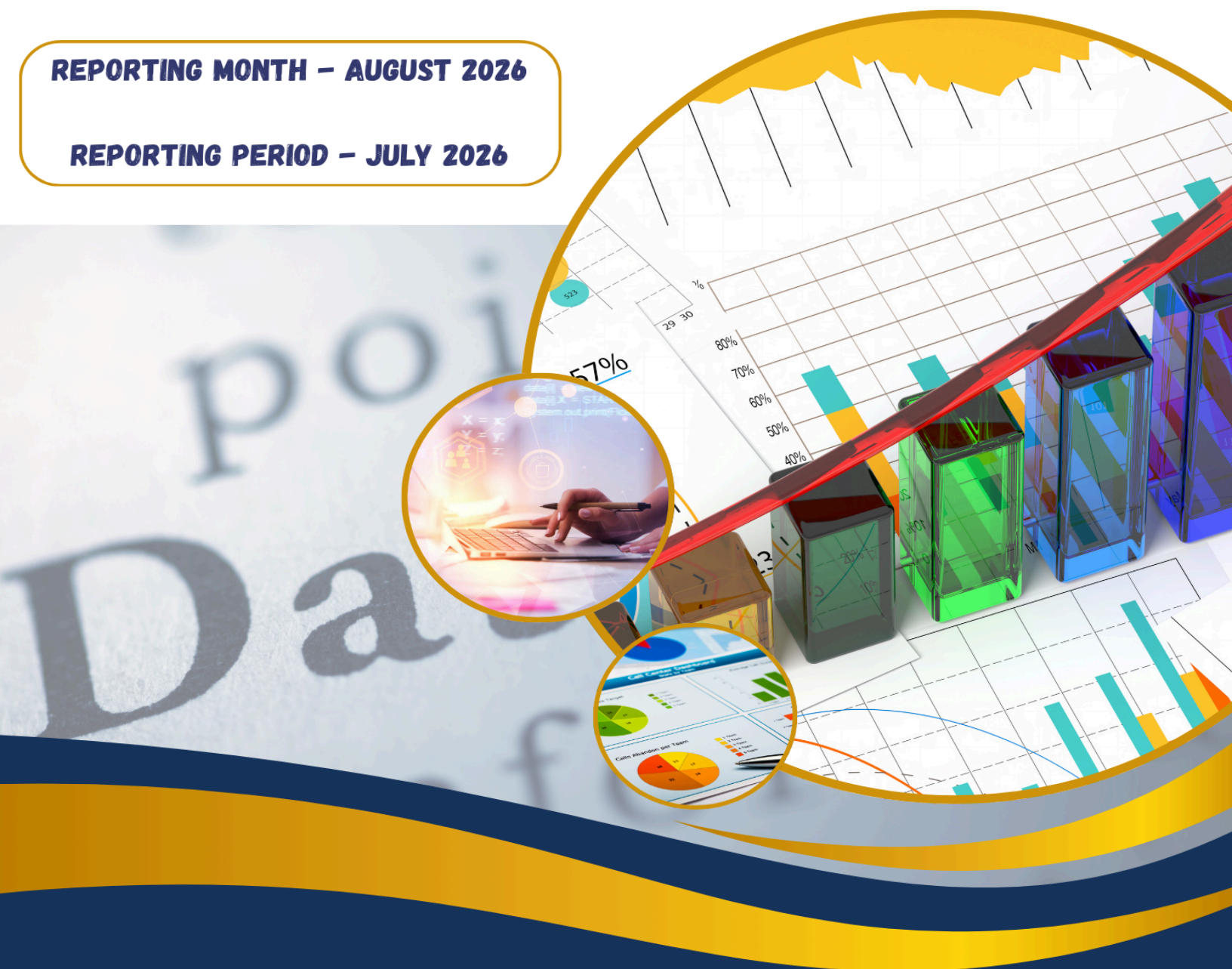


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HIV PREVENTION

Key Insights: HIV Prevention

Overall Programme Performance

- All prevention indicators fell below monthly targets across all populations and services, with year-to-date (YTD) achievements between 47%-72%, indicating several annual targets are at risk.
- HIV testing coverage remains strong, but linkage to and continuation of prevention services (e.g., PrEP and HIV self-testing) are lacking.

Population-Level Performance

Key Populations

- **Men Who Have Sex with Men (MSM):**
 - Reach: 54% of monthly target; YTD achievement: 47% reach, 49% testing; positivity yield of 3%.
 - July syphilis yield was 15% and YTD 11%.
 - Highest PrEP initiations for the quarter (30).
- **Transgender Women (TG):**
 - Highest positivity yield (5% YTD); Reach and test 79%; received 0 self-test kit.
 - July syphilis yield was 35% and YTD 17%.
- **Female Sex Workers (FSW):**
 - Coverage: 82% reach, 88% testing; YTD: 71% and 72%; positivity yield: 0.6%; 4 PrEP initiations.
- **At-Risk Youths (15-24 years):**
 - Underperformance: Reach: 77%, Testing: 81%; YTD: 60% for reach and 62% for test; low HIV yield (0.1%).
- **Sexually Active Adults (25-49 years):**
 - Males and females: Reach/Testing around 65%-88%; YTD HIV yield: 0.3% and 0.2%. respectively.
- **Adults 50+:**
 - YTD achievement: 50%-60%; both with HIV yield of 0.4 %.

Risk Reduction Trends

Risk Behavior Identified:

- **Common Behaviours:**
 - Incorrect/inconsistent condom use, multiple partners, inconsistent contraceptive use; over 267,364 condoms distributed.
- **Most Addressed Topics:**
 - Focus on condom use, HIV knowledge, stigma and discrimination.

PrEP Cascade Findings

- **High-Risk Individuals:**
 - 62% engaged in PrEP discussions, 5% referred; strong initiation rates (70% linkage, 100% initiation). Main weakness: early identification and referral.
- **National PrEP Cascade:**
 - Notable drop-offs between engaged and referred persons; data inconsistencies point to documentation issues.

HIV Self-Testing Insights

- Low distribution among key populations this month; missed prevention opportunities.

Major Strategic Concerns

1. Persistent underachievement across targets.
2. Weak PrEP referral pipeline with high drop-offs before linkage.
3. Critically low HIV self-test distribution needing urgent action.
4. Data quality issues affecting linkage and initiation reporting.
5. High positivity in TG and MSM, necessitating priority focus.

Priority Recommendations

1. Enhance outreach to MSM and TG populations due to high positivity yields.
2. Strengthen PrEP referral systems and follow-up mechanisms.
3. Scale up HIV self-test kit distribution among key populations.
4. Improve DHIS2 data quality and staff training.
5. Expand targeted interventions for condom use and multiple partnerships.
6. Integrate mental health, violence reduction, and STI care messaging.
7. Strengthen engagement strategies for older adults and sexually active males.

Outreach Testing Programme

Table 1 reflects the achievements of the Outreach Testing Programme toward monthly and year-to-date (YTD) targets across various population groups:

Key Populations:

- For MSM (men who have sex with men), reach was below the monthly target, achieving 54%, and test was also below the monthly target at 58%. This brings the annual achievements to 47% for reach and 49% for tests. The MSM HIV positivity rate for YTD was 3%.

In syphilis testing MSM, a total of 371 tests were conducted for the month, out of which 57 individuals tested reactive, resulting in a positivity yield of 15%. A total of 3,747 test were conducted YTD with 402 (11%) reactive.

- TG (transgender women) did not meet the monthly targets, achieving 79% for reach and test. YTD performance at the end of July stands at 68% for reach and 64% for test. TG continues to achieve the highest yield with 5% YTD.

In syphilis testing TG, a total of 23 tests were conducted for the month, out of which 8 individuals tested reactive, resulting in a positivity yield of 35%. YTD 223 were tested with 37 (17%) reactive.

- FSW (female sex workers) performance did not meet the monthly targets for both reach and test, with reach and test achieving 82% and 88%, respectively. This brings the YTD achievements for reach 71% and 72% with a YTD yield of 0.6%.

In syphilis testing of FSW, a total of 692 tests were conducted for the month, out of which 18 individuals tested reactive, resulting in a positivity yield of 3%. YTD 6601 tests were conducted with 139 reactive with a yield of 2%.

At Risk Youth (15-24 years):

- This group did not meet the monthly targets, with achievement for reach at 77% for and for test at 81% for July. This brings their YTD performance to 60% for reach and 62% for test, with a low YTD yield of 0.1%.
- Syphilis testing, a total of 1,838 tests were conducted for the month, out of which 10 individuals tested reactive, resulting in a positivity yield of 0.5%. YTD 16,227 tests were conducted with 80 reactive with a yield of 1%.

Sexually Active Females (25-49 years):

- Sexually Active Females (SAF) performed below the monthly target, with reach achievement at 82% and test at 88%, while YTD performances were 63% for reach and 66% for test. The HIV positivity yield YTD was 0.2%.

- In syphilis testing, a total of 3,225 tests were conducted for the month, out of which 37 individuals tested reactive, resulting in a positivity yield of 1%. YTD 28,376 tests were conducted with 239 reactive with a yield of 1%.

Sexually Active Males (25-49 years):

- Sexually Active Males (SAM) performed below the monthly target, achieving 65% for reach and 66% for test, with a YTD performance of 52% for both reach and 51% test. The YTD yield was 0.3%.
- In syphilis testing, a total of 1,708 tests were conducted for the month, out of which 24 individuals tested reactive, resulting in a positivity yield of 1%. YTD 15,444 tests were conducted with 129 reactive with a yield of 1%.

Female (50+ years):

- Females 50+ performed just below the monthly targets, achieving 92% for reach and 98% for test. This brings their YTD performance to 59% for reach and 60% for tests. The YTD yield was 0.4%.
- In syphilis testing, a total of 628 tests were conducted for the month, out of which 22 individuals tested reactive, resulting in a positivity yield of 4%. YTD 4,529 tests were conducted with 177 reactive with a yield of 4%.

Males (50+ years):

- Males 50+ did not meet the monthly targets, achieving 73% for reach and 72% for test. The achievement against the annual target is 50% for both reach and test, with a yield of 0.4%.
- In syphilis testing, a total of 470 tests were conducted for the month, out of which 12 individuals tested reactive, resulting in a positivity yield of 3%. YTD 3,815 tests were conducted with 120 reactive with a yield of 3%.

Table 1: HIV Reach & Test Achievements and Yields as of July 2026

Indicator	Monthly Target	Achievement July	% Achievement of Monthly Target	Annual Target (Jan-Dec 2026)	YTD Performance (Jan- Jul 2026)	% Achievement of YTD Target	Comments
MSM reached	714	382	54%	8,573	4,039	47%	Monthly target not met, annual target behind
MSM tested HIV	643	375	58%	7,716	3,843	49%	Monthly target not met, annual target behind
HIV Yield	13 (3.5%)			95 (3%)			
MSM tested Syphilis	371			3,747			
Syphilis Yield	57 (15%)			402 (11%)			
TG reached	33	26	79%	390	268	68%	Monthly target not met, annual target on track
TG tested HIV	29	23	79%	351	228	64%	Monthly target not met, annual target on track
Yield	1 (4.3%)			11 (5%)			
TG tested Syphilis	23			223			
Syphilis Yield	8 (35%)			37 (17%)			
FSW reached	877	724	82%	10,522	7,518	71%	Monthly target not met, annual target on track
FSW tested HIV	789	695	88%	9,470	6,867	72%	Monthly target not met, annual target on track
Yield	4 (0.6%)			39 (0.6%)			
FSW tested Syphilis	692			6601			
Syphilis Yield	18 (3%)			139 (2%)			
15-24 ARY reached	2,511	1,921	77%	30,134	18,135	60%	Monthly target not met, annual target on track
15-24 ARY tested for HIV	2,260	1,838	81%	27,120	16,706	62%	Monthly target not met, annual target on track
Yield	4 (0.2%)			25 (0.1%)			
ARY tested Syphilis	1,838			16,227			
Syphilis Yield	10 (0.5%)			80 (1%)			
25-49 SAF reached	4,098	3,353	82%	49,181	31,155	63%	Monthly target not met, annual target on track
25-49 SAF tested HIV	3,689	3,228	88%	44,263	29,253	66%	Monthly target not met, annual target on track
Yield	7 (0.2%)			73 (0.2%)			
25-49 SAF tested Syphilis	3,225			28,376			
Syphilis Yield	37 (1%)			239 (1%)			
25-49 SAM reached	2,861	1,859	65%	34,331	17,721	52%	Monthly target not met, annual target behind
25-49 SAM tested HIV	2,575	1,709	66%	30,898	15,841	51%	Monthly target not met, annual target behind
Yield	4 (0.2%)			45 (0.3%)			
25-49 SAM tested Syphilis	1,708			15,444			
Syphilis Yield	24 (1%)			129 (1%)			
50+ Females Reached	713	657	92%	8,550	5,060	59%	Monthly target not met, annual target on track
50+ Females Tested	641	628	98%	7,695	4,635	60%	Monthly target not met, annual target on track
Yield	2 (0.3%)			19 (0.4%)			
50+ Females tested Syphilis	628			4,529			
Syphilis Yield	22 (4%)			177 (4%)			
50+ Males Reached	721	525	73%	8,652	4,336	50%	Monthly target not met, annual target behind
50+ Males Tested	649	470	72%	7,787	3,885	50%	Monthly target not met, annual target behind
Yield	4 (0.9%)			15 (0.4%)			
50+ Male tested Syphilis	470			3,815			
Syphilis Yield	12 (3%)			120 (3%)			

Global Fund Prevention Indicators

Global Fund Result Based Contracting Indicators will be reported in August due to the revision of the targets.

HIV/STI Risk Behaviours Identified

The main risk behaviours identified in the month of July were incorrect or inconsistent condom use (4,925), multiple sex partners (699), substance misuse (248), inconsistent use of contraceptive methods (164) and history of STIs (126). Among condom distribution and risk behaviour, the highest was incorrect or inconsistent condom use, with 32,664 pieces distributed. Year-to-date, the main risk behaviours identified for which condoms were distributed are incorrect or inconsistent condom use (267,364), multiple sex partners (67,039) and inconsistent use of contraceptive methods (38,979) (Table 2).

Table 2: Risk Behaviour Identified and Condom Distribution for July 2026

Risk Behaviour Identified	July	Condoms Distributed	YTD Condom Distributed
Incorrect or Inconsistent Condom Use	4,925	32,664	267,364
Multiple Sex Partners	699	4,772	67,039
Substance Misuse	248	1,732	16,592
Inconsistent Use of Contraceptive Methods	164	1,548	38,979
History of STIs	126	991	12,069

The main risk conversations had in July 2026 were condom and lubricant use (9,466), knowledge (9,444), stigma and discrimination (4,171), partner reduction (2,349), seeking care for STIs (1,499), substance misuse (1,235) and abstinence (1,145). Family Planning (1,116), Anal genital care (1,039) and Violence reduction (802), were the least frequently addressed risk-reduction topics. Year-to-date, the highest-performing risk-reduction conversations focused on condom and lubricant use (88,172), knowledge (87,897), and stigma and discrimination (33,969) (Table 3).

Table 3: Risk Reduction Conversations for July 2026

Risk Reduction Conversations	No. of Interaction	YTD No. of Interaction
Condom and Lubricant Use	9,466	88,172
Knowledge	9,444	87,897
Stigma and Discrimination	4,171	33,969
Partner Reduction	2,349	20,914
Seeking Care for STIs	1,499	12,691
Substance Abuse	1,235	9,896
Abstinence	1,145	9,617
Family Planning	1,116	10,623
Anal / Genital care	1,039	8,477
Violence Reduction	802	6,428

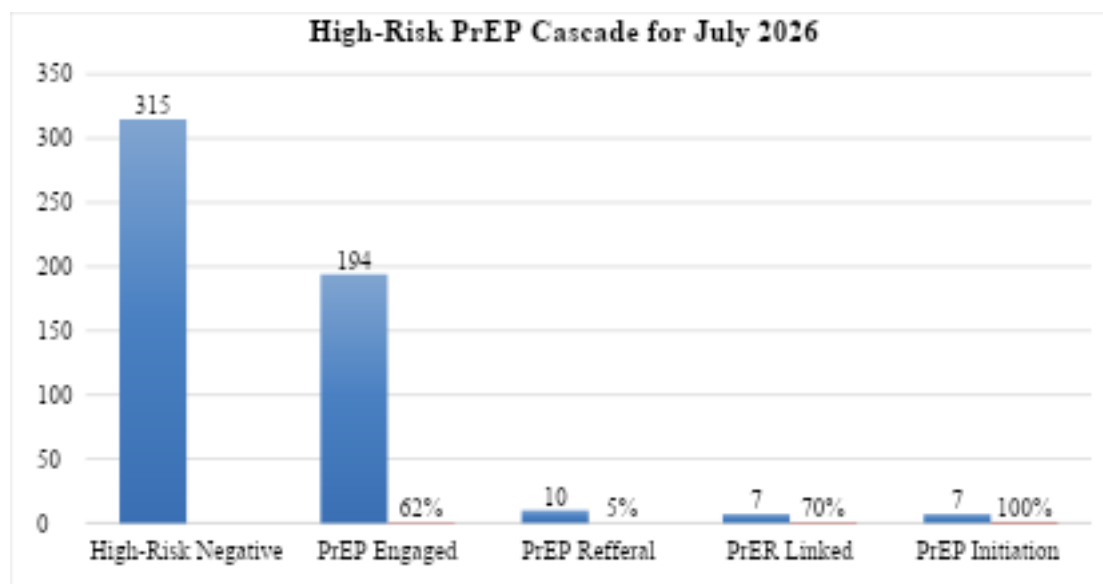


Figure 1: High Risk Population PrEP Cascade - July 2026

Figure 1 shows the PrEP Cascade for persons who reported high-risk behaviours (history of STIs and substance abuse) and tested negative for HIV. A total of 315 persons who reported high-risk behaviours tested negative for HIV during the month of July 2026. One hundred and ninety-four (62%) were engaged in discussions about PrEP, and 10 (5%) were referred to PrEP services. Seven persons were linked, representing 70%, and all 7 were initiated on PrEP, representing 100%.

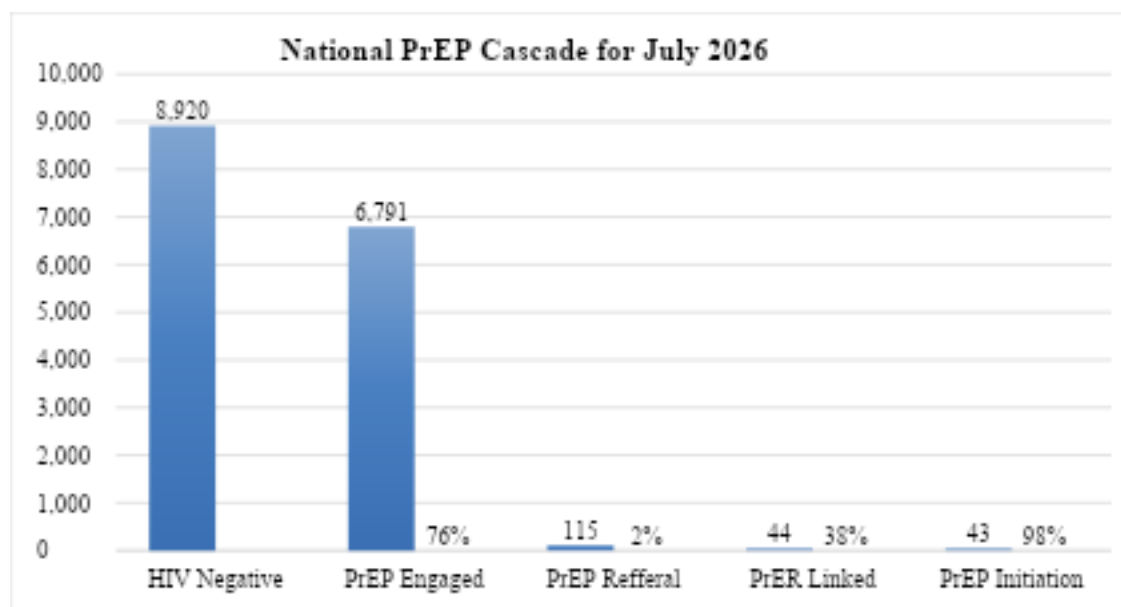


Figure 2: National PrEP Cascade July 2026

The PrEP Cascade tracks the continuum of care for people eligible for PrEP. It begins with 8,920 HIV-negative individuals and shows a progressive decline across subsequent stages: 6,791 (76%) were engaged, 115 (2%) were referred, and 44 (38%) were linked, highlighting the data entry gap with the inconsistent entries for linkage. Subsequently, 43 (98%) were initiated on PrEP (Figure 2).

Population PrEP distribution based on data obtained from DHIS2 for July 2026 resulted in a total of 8,920 persons tested negative for HIV. Of these, 6,791 were engaged with PrEP information, and 115 were documented as referred for PrEP services. Subsequently, 44 persons were linked to PrEP services, and 43 were initiated on PrEP. MSM accounted for the highest number of PrEP initiations 30 (70%), followed by FSW 4 (9%) and ARY individuals 4 (9%). No initiation was reported among TG. Data entry inconsistencies resulted in a higher number of persons being documented as linked than were referred to PrEP services in the MSM population. (Table 4).

Table 4: PrEP Uptake for July 2026

Population	HIV Negative	Engaged	Referred	Linked	Initiated
SAF	3219	2548	30	3	2
ARY	1834	1452	17	4	4
SAM	1705	1354	13	1	1
FSW	688	473	26	4	4
M50+	466	356	2	1	1
F50+	624	412	3	1	1
MSM	362	183	24	30	30
TG	22	13	0	0	0
Total	8,920	6,791	115	44	43

HIV Self Testing

During July 2026, a total of 205 HIV self-test kits were distributed. Of these, 1,94 were distributed through the unassisted method, while 11 were distributed through the assisted method. The distribution of kits by sex was higher among females, 106 were unassisted and 6 were assisted, while among males, 88 were unassisted and 5 were assisted. The highest distribution among females was observed in the 20-39 age group, while among males it was concentrated in the 25-44 age group. No reactive results were reported for either testing method (Table 5).

Table 5: HIV Self-Test distribution for July 2026						
Sex/Age Group	Assisted	Assisted Non-Reactive	Assisted Reactive	Unassisted	Unassisted Non-Reactive	Unassisted Reactive
Female	6	0	0	106	32	0
16 - 19	0	0	0	5	3	0
20 - 24	0	0	0	17	6	0

Table 5: HIV Self-Test distribution for July 2026						
Sex/Age Group	Assisted	Assisted Non-Reactive	Assisted Reactive	Unassisted	Unassisted Non-Reactive	Unassisted Reactive
25 - 29	5	0	0	24	4	0
30 - 34	1	0	0	25	3	0
35 - 39	0	0	0	12	4	0
40 - 44	0	0	0	8	3	0
45 - 49	0	0	0	9	5	0
50+	0	0	0	6	4	0
Male	5	5	0	88	19	0
16 - 19	0	0	0	6	2	0
20 - 24	1	1	0	9	0	0
25 - 29	1	1	0	14	2	0
30 - 34	1	1	0	19	5	0
35 - 39	1	1	0	15	2	0
40 - 44	1	1	0	12	4	0
45 - 49	0	0	0	2	1	0
50+	0	0	0	11	3	0
Total	11	5	0	194	51	0

HIV/ST kits distribution was low for the key populations, with a total distribution of 38 (FSW 31, MSM 7 and TG 0). There were no reported reactive results, whether assisted or unassisted (Table 6).

Table 6: Key Population HIV Self-Test Kits Distribution for July 2026

Table 5: Self-Test Distribution for Key Population (Assisted and Unassisted) for July 2026							
Key Population	Assisted	Assisted Non-Reactive	Assisted Reactive	Unassisted	Unassisted Non-Reactive	Unassisted Reactive	Total
FSW	6	0	0	25	1	0	31
MSM	5	5	0	2	0	0	7
TG	0	0	0	0	0	0	0

Total	11	5	0	27	1	0	38
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Key Population programme Quality Indicators

During July 2026, HIV prevention services reached 724 FSW, 382 MSM, and 26 TG individuals. Of those reached, 19% of TG, 14% MSM and 9% of FSW individuals presented with high-risk behaviours. With respect to referrals to other health services, 27% of TG individuals, 9% of MSM, and 2% of FSW received referrals. The HIV positivity yield from testing was 4% TG, 3% for MSM and 0.6% for FSW. Regarding HIV self-testing, 86% of self-test kits were distributed to MSM, and 68% distributed to FSW were for primary use. For secondary use, 32% of kits were distributed to FSW and 14% distributed to MSM. There were no HIV Self-test kits distributed to high-risk individuals. Regarding PrEP services among high-risk individuals, 13% of MSM, 0% of FSW, and 0% of TG individuals were linked to PrEP services. Additionally, 13% of MSM received PrEP products (Table 7).

Table 7:Key Population Quality Indicators for July 2026

Indicators	Population	Achievement %
Percentage of key populations reached who were high-risk (History of STIs or Substance Misuse).	MSM	14% (54/382)
	FSW	9% (66/724)
	TG	19% (5/26)
Percentage of key population who received prevention basic services and were referred to other health services (e.g. psychological support, treatment for STIs or treatment care and support if HIV reactive)	MSM	9% (34/382)
	FSW	2% (11/724)
	TG	27% (7/26)
Percentage of key populations tested positive out of the total number tested for the reporting period (positivity yield).	MSM	3% (13/375)
	FSW	0.6% (4/695)
	TG	4% (1/23)
Percentage of self-test kits distributed for primary use (giving kits to the OVP member) to key populations within the reporting period.	MSM	86% (6/7)
	FSW	68% (21/31)
	TG	0% (0/0)
Number of self-test kits distributed to key populations for secondary use (to the OVP partner(s)) within the reporting period.	MSM	14% (1/7)
	FSW	32% (10/31)
	TG	0% (0/0)
Percentage of high-risk key populations (History of STIs or Substance Misuse) receiving self-tests for primary use within the reporting period. Per population	MSM	0% (0/0)
	FSW	0% (0/0)
	TG	0% (0/0)

Indicators	Population	Achievement %
Percentage of high-risk (History of STIs or Substance Misuse) key populations receiving self-tests for secondary use within the reporting period. Per population	MSM	0% (0/0)
	FSW	0% (0/0)
	TG	0% (0/0)
Percentage of key population at high-risk (History of STIs or substance misuse) receiving any PrEP product during the reporting period and are registered in the DHIS2.	MSM	13% (6/48)
	FSW	0% (0/63)
	TG	0% (0/4)
Percentage of key populations at high-risk (History of STIs or substance misuse) linked to PrEP services.	MSM	13% (6/48)
	FSW	0% (0/63)
	TG	0% (0/4)

Private Lab Monitoring

The SI Component continues to engage major private laboratories to provide monthly HIV reports. During the reporting period, two laboratories (Sagikor, Guardian Life) submitted their report on time. A total of 367 HIV tests were conducted, with 42% (n=155) of the cases being males and 58% (n=212) females. Notably, there were zero positive cases reported (Table 8).

The SI unit, through our monthly engagements, continues to remind these private labs that the submission of the class one notification form to the National Surveillance Unit is mandatory once a case is detected.

Table 8: HIV Monthly Reports from Private Labs - July 2026

Laboratory	Report Received:	No. Tested		Negative Results		Positive Results		Inconclusive Results	
		No. of males	No. of females	No. of males	No. of females	No. of males	No. of females	No. of males	No. of females
Sagikor	Yes	131	181	131	181	0	0	0	0
Guardian Life	Yes	24	31	24	31	0	0	0	0
Andrew's Memorial Hospital	No	-	-	-	-	-	-	-	-
Central Medical	No	-	-	-	-	-	-	-	-
Biomedical	No	-	-	-	-	-	-	-	-
Hargreaves	No	-	-	-	-	-	-	-	-
Microlabs Limited	No	-	-	-	-	-	-	-	-
Medical Associates	No	-	-	-	-	-	-	-	-
Eagles Medical	No	-	-	-	-	-	-	-	-
Consolidated	No	-	-	-	-	-	-	-	-
Total		155	212	155	212	0	0	0	0

PREVENTION OF MOTHER TO CHILD TRANSMISSION (PMTCT)

Key Insights: PMTCT

1. HIV Testing Coverage Among Pregnant Women Remained Exceptionally High

- **99%** (1,335/1,341) of pregnant women who delivered were tested for HIV.
- **St. Mary, Portland, St. Ann, St. Catherine, St. Thomas, Manchester, Westmoreland and Hanover** achieved **100% HIV status awareness**.
- **St. James and Trelawny** each achieved **99%** coverage.
- The high level of HIV testing coverage continues to support early identification and intervention to prevent mother-to-child transmission (MTCT).

2. PMTCT Reporting Compliance Remains a Significant Gap

- National reporting compliance was **69%** (9/13 expected reports).
- The **Western and North East Regions** achieved **100%** reporting compliance.
- The **Southern and South East Regions** each achieved only **33%** reporting compliance.
- Only **62%** of expected reports were received on time.
- Data from **Kingston and St. Andrew, St. Elizabeth and Clarendon** were not submitted, limiting the completeness and representativeness of the national analysis.

3. Strong PMTCT Maternal and Infant Outcomes

- **18 HIV-positive pregnant women** were identified during the reporting period.
- **17 women (94%)** received HAART during pregnancy.
- One mother presented at delivery while lost to follow-up and did not receive ARVs during pregnancy.
- **One stillbirth** was reported.
- All **17 HIV-exposed infants (100%)** received ARV prophylaxis within **72 hours of birth**, demonstrating strong adherence to infant prophylaxis protocols.

4. Syphilis Screening Coverage Remained High

- **99%** (1,338/1,338) of women who delivered were tested for syphilis.
- **Seven women** tested positive, representing a **0.5% positivity rate**.

5. Maternal Syphilis Treatment Requires Improvement

- Only **6 of 7 (86%)** syphilis-positive women received adequate treatment at least **30 days before delivery**.
- **One woman (14%)** did not receive adequate treatment within the recommended timeframe, representing a missed opportunity to prevent vertical transmission.

6. No Congenital Syphilis Cases Reported

- **No cases of congenital syphilis** were reported during the reporting period.
- While this is a positive outcome, continued attention to timely maternal treatment is necessary to sustain progress toward elimination of mother-to-child transmission of syphilis.

Strengths

- Near-universal HIV and syphilis testing among pregnant women (**99% coverage**).
- **100% ARV prophylaxis coverage** among reported HIV-exposed infants within 72 hours of birth.
- High HAART coverage (**94%**) among HIV-positive pregnant women.
- **100% HIV status awareness** achieved across eight reporting parishes.
- No congenital syphilis cases were reported.

Priority Areas for Improvement

- Increase PMTCT reporting compliance, particularly in the **Southern and South East Regions**.
- Improve the **completeness and timeliness** of PMTCT reporting, particularly from parishes with outstanding submissions.
- Strengthen **retention and follow-up** of pregnant women living with HIV to prevent late presentation and missed opportunities for antenatal ART.
- Ensure all women diagnosed with syphilis receive **adequate treatment at least 30 days before delivery**.
- Strengthen follow-up of untreated or late-treated syphilis cases to sustain progress toward **elimination of mother-to-child transmission**.

A review of the country's PMTCT data for July indicates that HIV testing was offered to 1,335 of the 1,341 pregnant women who delivered, resulting in a testing coverage rate of 99%. Among the parishes that reported, St. Mary, Portland, St. Ann, St. Catherine, St. Thomas, Manchester, Westmoreland and Hanover achieved 100% HIV status awareness among pregnant women. St. James and Trelawny recorded slight gaps, with 99% of pregnant women knowing their HIV status, respectively (see Figure 3).

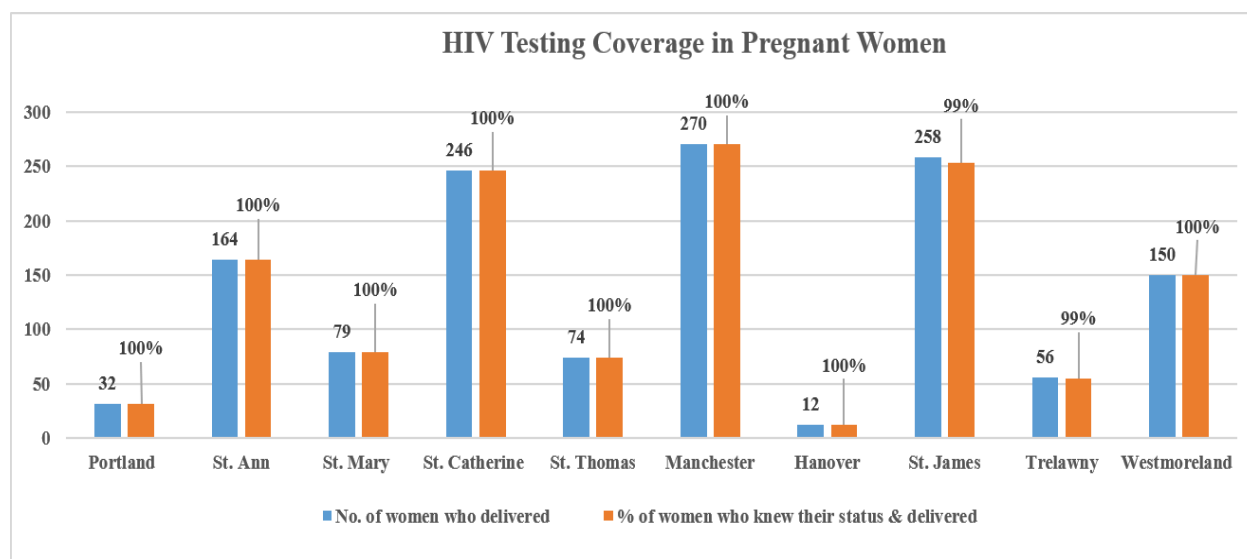


Figure 3: HIV Testing in Pregnant Women – July 2026

PMTCT Reporting Compliance

It is important to note that data from the parishes of Kingston and St. Andrew, St. Elizabeth and Clarendon were not submitted. Therefore, the current analysis reflects only 69% of the expected data from the Regional Health Authorities for the reporting period (Table 10).

Table 9: Reporting Compliance for PMTCT datasets – July 2026

Region	Expected Reports	Actual Reports	Percent Received	Reports On Time	Percentage On Time
Western Region	4	4	100%	4	100%
North East Region	3	3	100%	3	100%
Southern Region	3	1	33	1	33%

South East Region	3	1	33	0	0%
Total	13	9	69%	8	62%

Preventing Mother-Child-Transmission of HIV

A total of 18 HIV positive women were identified during the reporting period. Of these, 17 received highly active antiretroviral therapy (HAART) during pregnancy to reduce the risk of mother-to-child transmission (MTCT). One mother did not receive ARV medicine during pregnancy as she presented at delivery and was identified as LTFU; she later absconded post delivery. Notably, 1 stillbirth was recorded and all 17 HIV-exposed infants (HEIs), received antiretroviral (ARV) prophylaxis within 72 hours of birth (Figure 4).

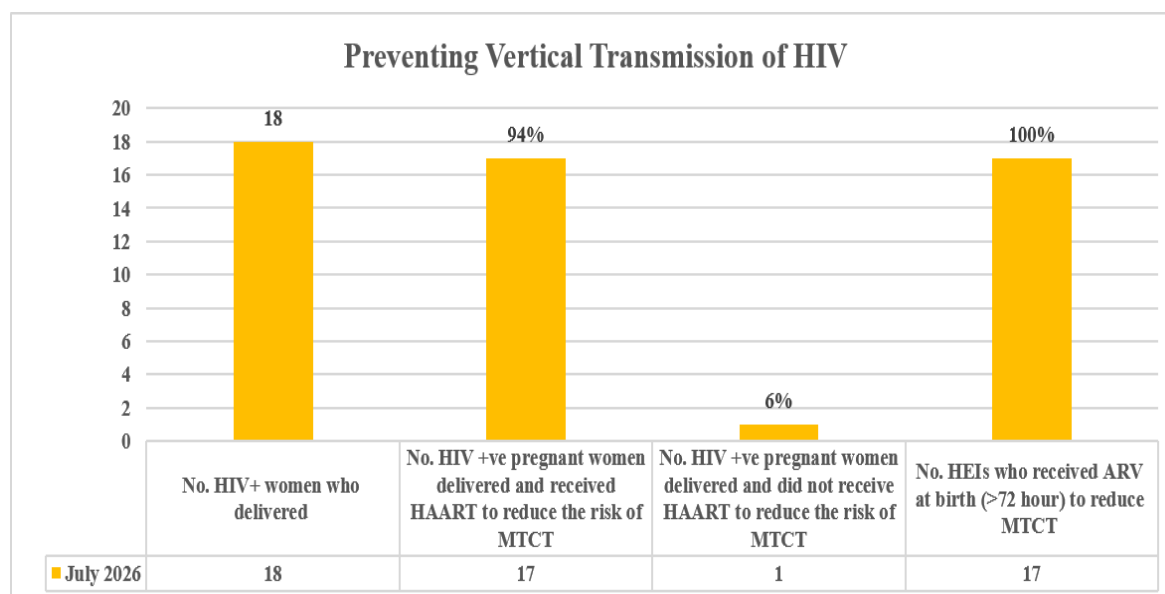


Figure 4: Preventing Mother-Child-Transmission of HIV- July 2026

Syphilis Testing in Pregnant Women Who Delivered

During the reporting period, 1,338 of the 1,338 women who delivered received a syphilis test, representing a testing coverage rate of 99%. A positivity rate of 0.5% (7 positive cases) was recorded. Notably, only 6 (86%) received adequate treatment at least 30 days prior to delivery. The remaining 1 (14%) did not receive adequate treatment at least 30 days prior to delivery. Additionally, no cases of congenital syphilis were reported (figure 5).

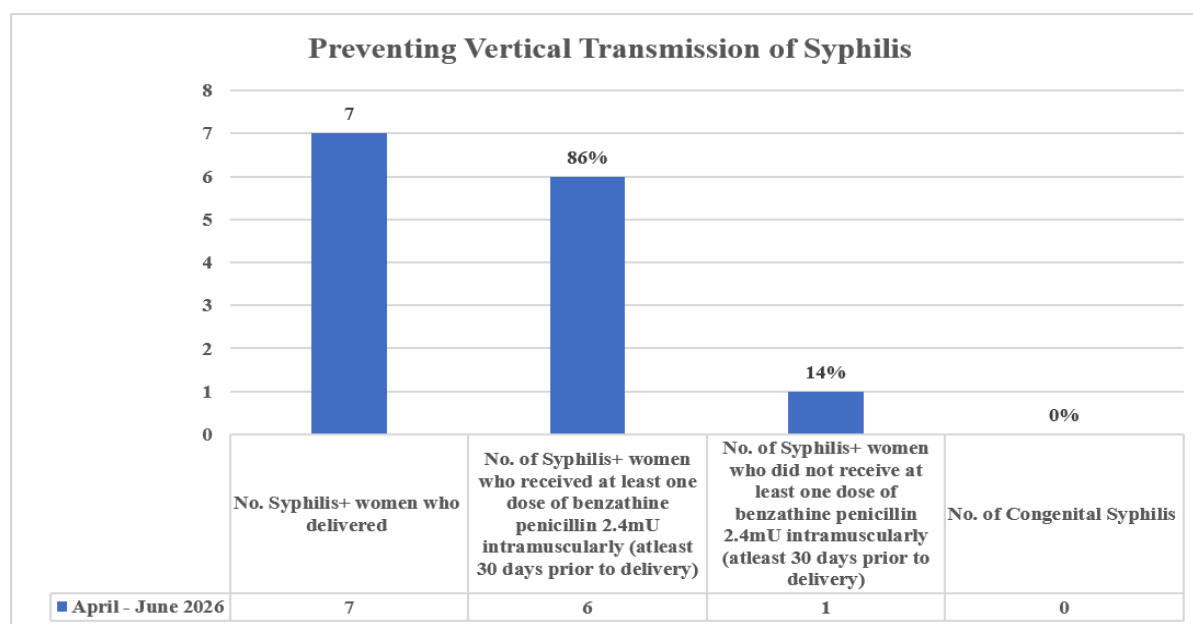


Figure 5: Preventing vertical transmission of Syphilis – July 2026

HIV TREATMENT

Key Points

- HIV confirmations decreased slightly:** 60 new HIV cases were confirmed in TSIS in July 2026, down from 63 in July 2025. YTD, **458 cases** were confirmed in 2026 compared with 467 in 2025.
- Most PLHIV have been diagnosed:** Jamaica has an estimated **28,000 PLHIV**, with approximately **27,000 (96%) diagnosed**. Linkage to care improved from 69% in June to **70% in July**.
- Major treatment cascade gaps remain:** Of those linked to care, **81% were retained in care**, **77% retained on ARV**, **67% had a valid viral load test**, and **61% were virally suppressed**. Viral suppression remains the largest challenge.
- The 95-95-95 cascade shows substantial gaps:** The first 95% target was **exceeded by 1%**, but there was a **39% gap for the second 95** and a **16% gap for the third 95**.
- Females performed slightly better across cascade indicators:** Retention in care was **82% for females vs. 81% for males**, retention on ARV was 78% vs. 76%, viral load testing was 68% vs. 65%, and viral suppression was 62% vs. 59%. Critical viral suppression underperformance occurred among **females aged 15–29 and males aged 35–39**.
- Linkage and ART initiation were strong in July:** 56 newly diagnosed PLHIV were linked to care, 68 were linked overall, and **60 initiated ART following linkage**. An additional 64 initiated ARV when previous-month linkages were included.
- Missing baseline CD4 data is a major concern:** **81% of people initiated on ART had no baseline CD4 recorded**. The report attributes this mainly to CD4 tests not being requested

by clinicians or completed by patients. Among those with results, 42% had CD4 >350, 33% had 200–350, and 25% had <200.

8. **Viral suppression within six months was 80% among eligible clients:** However, gaps in viral load testing may be related to clients being out of care, delays in receiving results, and missed opportunities for testing. The six-month period after ART initiation is currently being used as a proxy while database amendments are ongoing.
9. **Loss to follow-up and data quality are significant issues:** Of 3,641 LTFU clients, **69% (2,507) had no documented reason for LTFU**. The largest documented categories were unable to locate (420), in private care (276), and refusing care (231).
10. **Parish performance shows important variation:** No parish met the overall **95% viral suppression target**. St. Thomas and Trelawny were highest at 92%, while St. Elizabeth was lowest at 51%. For ARV retention, St. Elizabeth (98%) and Westmoreland (97%) performed strongest, although **no parish met its ARV target**.

Summary

July 2026 shows continued progress in HIV diagnosis, linkage to care, and ART initiation, but the major programme challenges remain retention, viral load testing, viral suppression, LTFU, and data completeness, particularly missing baseline CD4 information and undocumented reasons for LTFU.

HIV Surveillance

In July 2026, 60 cases of HIV were confirmed in TSIS, which is **3** fewer than in July 2025. See Table 10 below:

Table 10: Year-to-date (YTD) comparison of confirmed cases for 2026 and 2025 - July 2026

Month	Cases 2026	Cases 2025
January	59	86
February	71	51
March	83	70
April	67	67
May	69	73
June	49	57
July	60	63
Total YTD	458	467

National Treatment Cascade

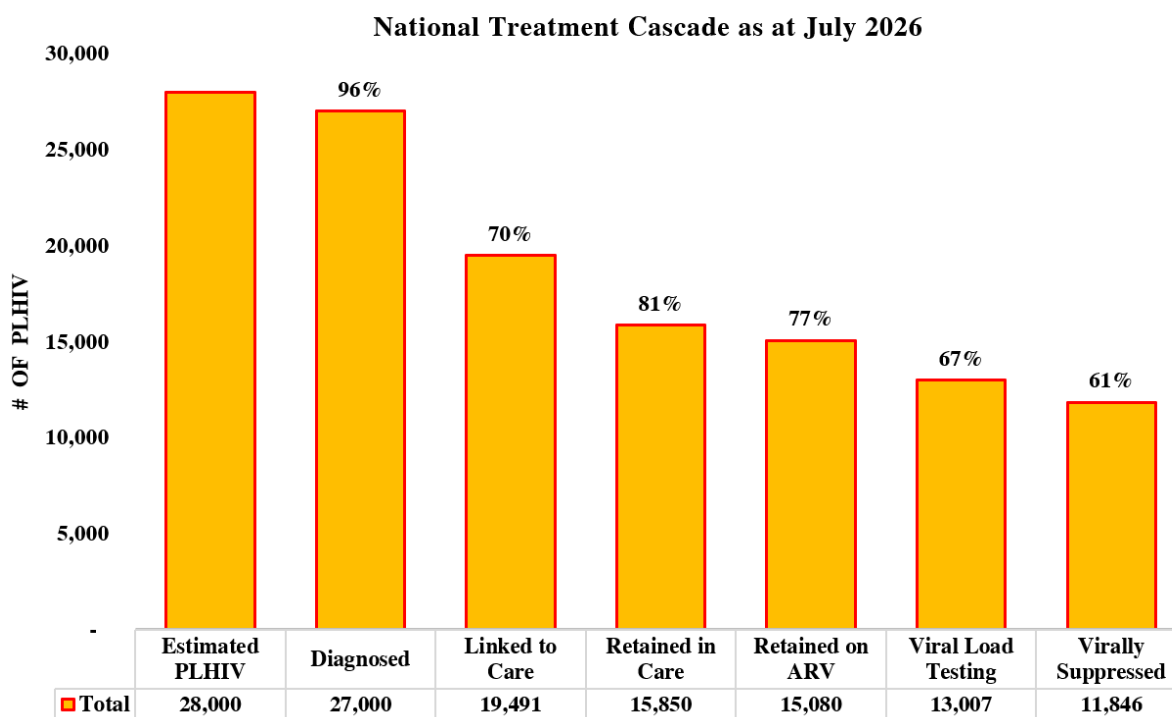


Figure 6: National Treatment Cascade July 2026

Based on the UNAIDS Spectrum 2025 estimates, approximately 28,000 persons are living with HIV (PLHIV) in Jamaica, of whom an estimated 27,000 (96%) have been diagnosed.

Compared with June 2026 (69%), the proportion of PLHIV linked to care increased by 1% in July 2026 (70%), although the absolute number increased from 19,434 to 19,491 PLHIV. This increase reflects 71 PLHIV newly linked and 31 PLHIV who returned to care during July 2026, offset by deaths (33), mergers (11) and one PLHIV removed due to a missing first clinic date.

Of those linked to care, 81% were retained in care, 77% retained on ARV, 67% with a valid viral load test and 61% virally suppressed.

National 95-95-95 Cascade

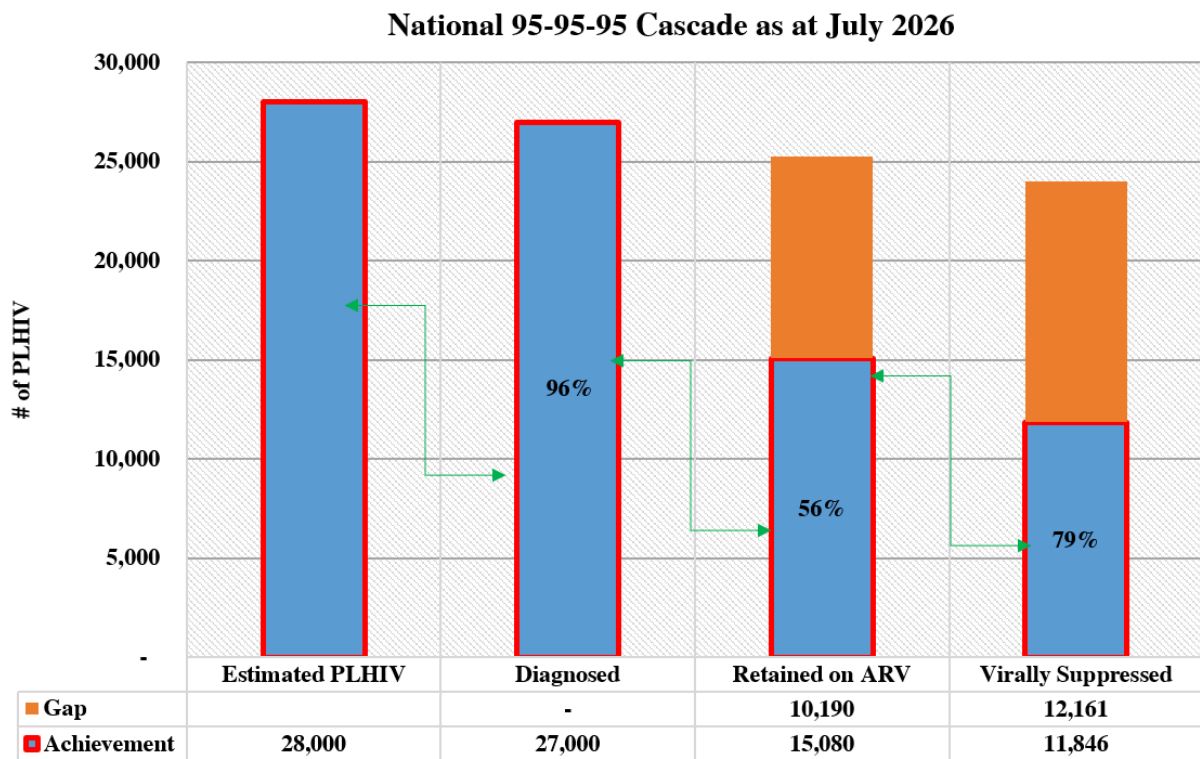


Figure 7: National 95-95-95 Cascade July 2026

The national 95-95-95 cascade indicates that the first gap has been overachieved by 1%, a **39%** gap for the second 95, and **16%** gap for the third 95 (Figure 7).

Sex and Age Specific Linked to Care Cascade

Target: 95%

Met/surpassed national target ($\geq 95\%$)
Moderate performance (70% - 94%)
Underperformance (50% - 69%)
Critical Underperformance ($\leq 50\%$)

Table 11: Specific Cascade Focus Data Disaggregated by Sex and Age as of July 2026

Age	Linked to Care		Retained in Care		Retained on ARV		Viral Load Testing		Viral Suppression	
	Male	Female	Male	Female	Male	Female	Male	Female	Male	Female
0-4	8	10	88%	100%	88%	100%	75%	100%	63%	70%
5-9	18	37	89%	97%	89%	89%	78%	78%	72%	68%
10-14	28	28	89%	100%	82%	93%	71%	75%	57%	61%
15-19	67	108	94%	87%	91%	81%	67%	68%	54%	48%
20-24	309	439	80%	77%	75%	71%	59%	54%	52%	44%
25-29	589	802	77%	74%	71%	68%	61%	56%	57%	49%
30-34	868	1,112	75%	73%	70%	69%	58%	56%	54%	50%
35-39	847	1,298	74%	77%	68%	74%	56%	64%	49%	57%
40-44	922	1,333	78%	79%	74%	76%	61%	65%	55%	60%
45-49	992	1,315	78%	84%	74%	82%	65%	73%	58%	68%
50-54	1,177	1,287	80%	88%	76%	84%	66%	74%	60%	69%
55-59	1,117	1,020	84%	87%	79%	85%	68%	77%	62%	72%
60-64	869	846	88%	88%	86%	84%	74%	76%	68%	71%
65-69	535	538	88%	90%	85%	87%	76%	77%	69%	71%
70+	494	474	88%	88%	83%	86%	74%	76%	66%	70%
Total	8,843	10,648	81%	82%	76%	78%	65%	68%	59%	62%

The sex and age disaggregated cascade continue to demonstrate slightly stronger performance among females across all indicators. Nationally, retention in care reached 82% among females and 81% among males, while retention on ART was 78% and 76%, respectively. Viral load testing remained below the national target for both sexes, achieving 68% among females and 65% among males, while

viral suppression remained the weakest indicator at 62% for females and 59% for males. Critical underperformance was seen for viral suppression for females 15-29 and males 35-39 (Table 11).

PLHIV Diagnosed, Linked and ARV Initiation

Table 12: Diagnosed, Linkage and ARV initiation- July 2026

Indicator	Males	Females	Total
PLHIV diagnosed AND linked to care in July 2026	24 (43%)	32 (57%)	56
PLHIV linked to care in July 2026 including PLHIV diagnosed in previous months	32 (47%)	36 (53%)	68
PLHIV linked to care AND initiated on ARV in July 2026	28 (47%)	32 (53%)	60
PLHIV initiated on ARV including PLHIV linked to care in previous months	30 (47%)	34 (53%)	64
PLHIV initiated on ARV prior to being linked to care	755 (42%)	1,049 (58%)	1,804

In July 2026, **females accounted for the majority across all indicators**, ranging from 53% to 58%. A total of **56 newly diagnosed PLHIV were linked to care**, while **68 PLHIV were linked to care overall**, including those diagnosed in previous months. **60 PLHIV initiated ART following linkage**, with **64 initiating ARV overall**. Notably, **1,804 PLHIV initiated ARV prior to being linked to care**, of whom **58% were female and 42% were male** (Table 12).

Of note, PLHIV initiated on ARV prior to being linked to care is an ongoing data verification process being conducted by the treatment sites to ensure the data is accurate and not due to data entry error.

Table 13: PLHIV Initiated on ARV in July 2026 (including PLHIV linked to care from previous months) disaggregated by Baseline CD4, Age and Sex

Age	Missing CD4		<200		200-350		>350		Total
	Male	Female	Male	Female	Male	Female	Male	Female	
15-19	3	3	0	1	0	0	0	0	7
20-24	0	4	0	0	0	0	0	1	5
25-29	6	5	0	0	0	0	1	0	12
30-34	1	5	0	0	2	0	0	0	8
35-39	3	0	0	0	1	0	0	0	4
40-44	2	2	0	0	0	0	0	0	4
45-49	2	5	0	0	0	0	2	0	9
50-54	0	5	2	0	0	1	0	0	8
55-59	1	0	0	0	0	0	1	0	2
60-64	2	0	0	0	0	0	0	0	2
65-69	1	0	0	0	0	0	0	0	1
70+	0	2	0	0	0	0	0	0	2
Total	21	31	2	1	3	1	4	1	64
%	33%	48%	3%	2%	5%	2%	6%	2%	100%

The table above aimed to monitor baseline CD4 at ARV initiation for July 2026. PLHIV with a baseline CD4 <200 was 5% of the total initiated on ART in July 2026 but were linked to care in previous months. The majority (81%) of PLHIV initiated on ART were not found to have a baseline CD4 entered in the database, which is of major concern as this limits the monitoring of the programme's impact for early initiation for HIV (Table 13).

Reasons for missing baseline CD4 were found to be the lack of CD4 test being requested by the clinicians and done by the patients. The treatment sites have been instructed to ensure PLHIV with a missing CD4 in the database has one done as soon as possible.

On a positive note, of the PLHIV that had a baseline CD4 result in the database, a higher proportion had a CD4 >350 (42%), compared with those with a CD4 200-350 (33%) and CD4 <200 (25%).

Viral Suppression Within 6 Months of Initiating ARVs (Monthly)

Figure 8 below shows the viral suppression among those eligible, 6 months post ARV initiation, for July 2026. Amendments to the database are ongoing to capture all criteria relating to viral load eligibility namely:

- six months after initiation on ARV
- six months after last unsuppressed viral load
- six months after first suppressed viral load
- twelve months for suppressed clients with two consecutive valid suppressed viral load
- > 3 months after re-initiation

Until then, the period of 6 months after ARV initiation is being used as a proxy. Viral Suppression was found to be 80% of those eligible for a viral load test.

Possible reasons for the gap between the viral load eligible column and the viral load testing column are PLHIV being out of care, delays in receiving viral load results and missed opportunities for viral load tests.

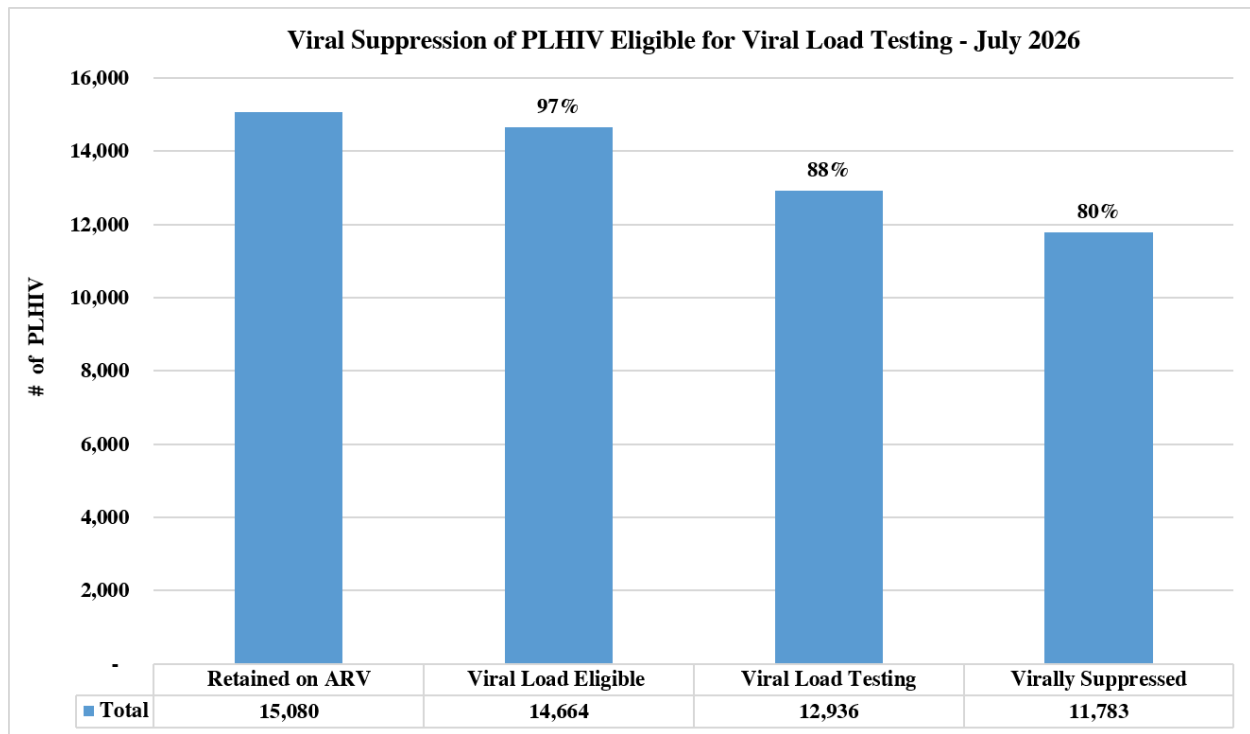


Figure 8: Viral Suppression Within 6 Months of Initiating ARVs – July 2026

Lost to Follow-Up

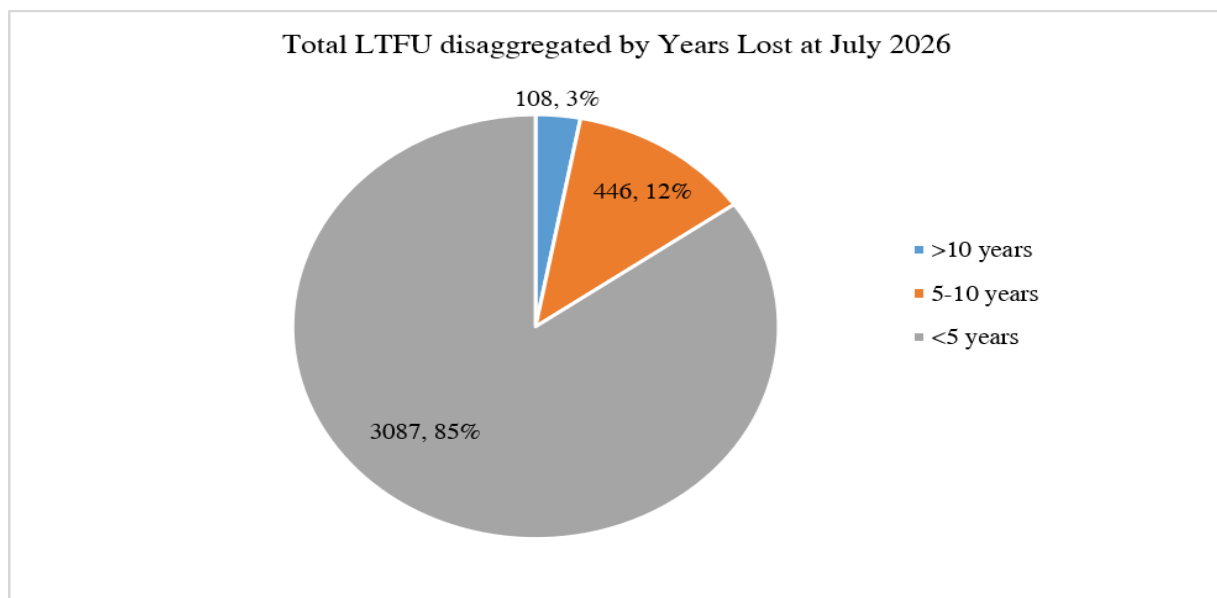


Figure 9: Total LTFU Disaggregated by Years - July 2026

The figure above indicates that lost-to-follow-up was highest in the <5 year period at 85%.

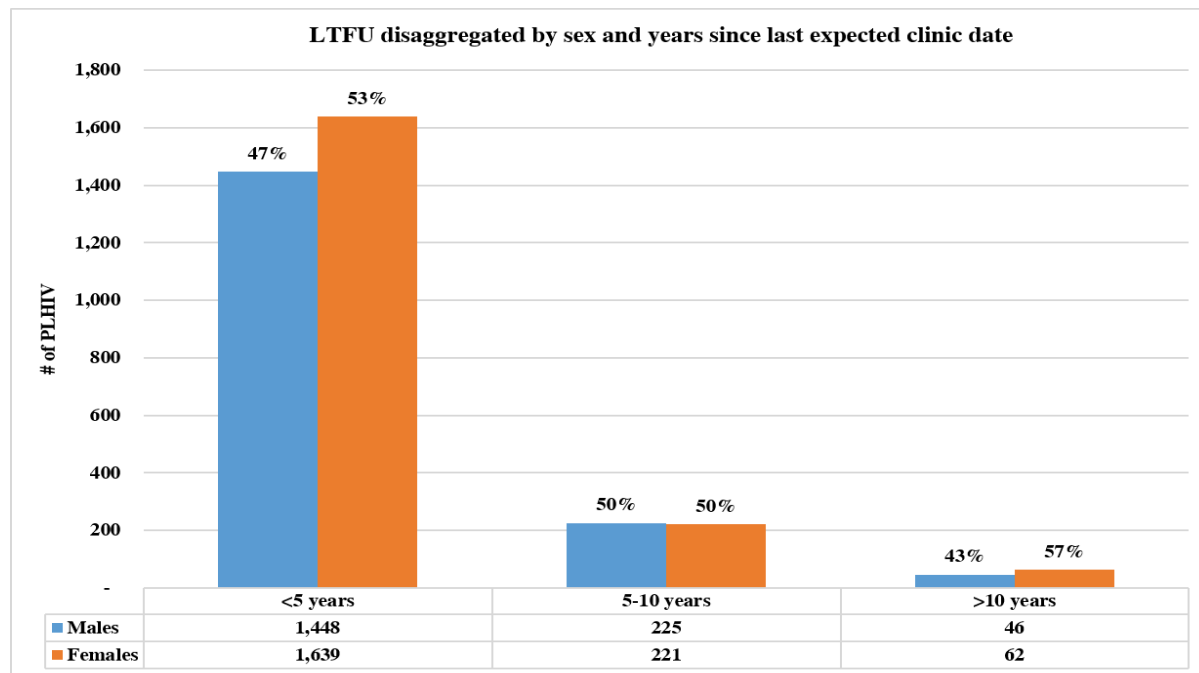


Figure 10: Total LTFU Disaggregated by Sex and Years - July 2026

The above figure highlights a higher proportion of females compared to males who are lost to follow-up less than 5 years (f: 53% v. m: 47%) and >10 years (f: 57% v. m: 43%). The proportion of females and males lost to follow up for 5 -10 years was the same (f: 50% v. m: 50%)

Table 14: Reason for LTFU entered in TSIS for the LTFU cohort - July 2026

Reasons for LTFU	# of PLHIV
Taking alternative medication	1
Change of address	3
Case closed	6
Returned to care	12
Changed clinic	22
Social issues	40
Migrated	53
Other	70
Refusing care	231
In private care	276
Unable to locate	420
Reason for LTFU missing	2,507

Grand Total	3,641
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The table above summarizes the reasons for PLHIV being classified as lost to follow-up (LTFU) based on information entered into the TSIS2 database by the treatment sites. A major data quality concern is that 69% (2,507) of LTFU cases have no documented reason, limiting the ability to accurately identify barriers to retention and implement targeted interventions.

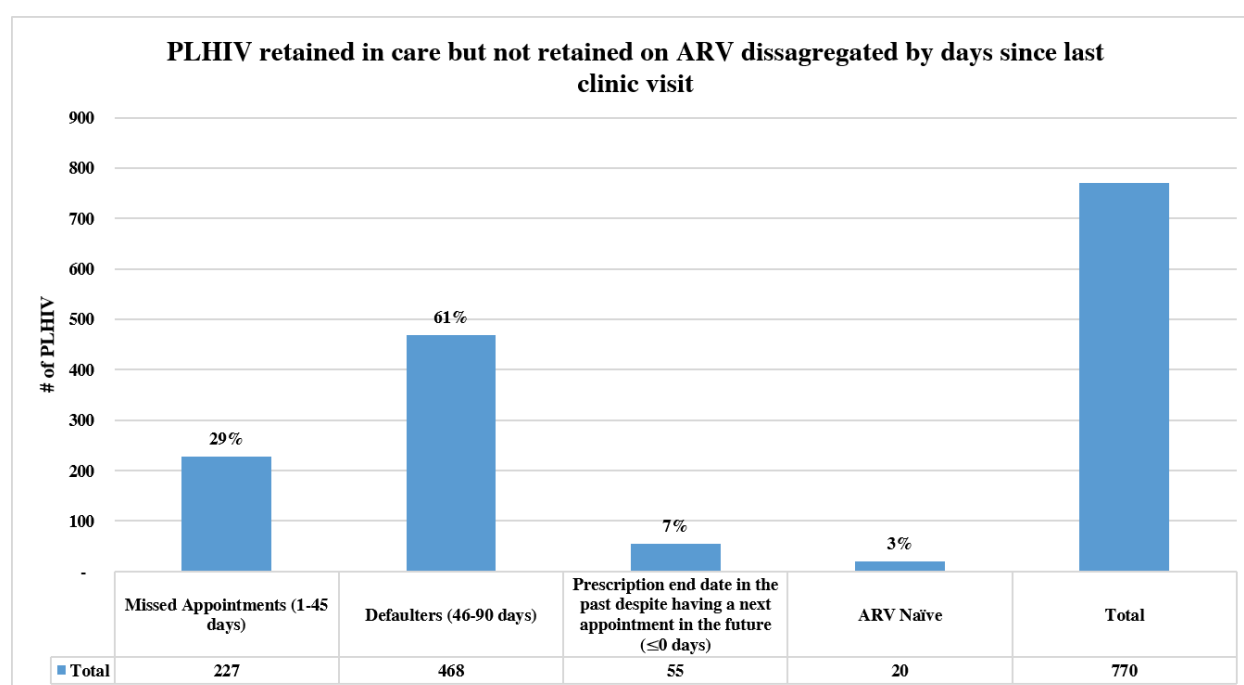


Figure 11: Disaggregated Days Since Last Appointment Not Retained on ARV – July 2026

The figure above shows an examination of the PLHIV retained in care but not currently on ARV at the end of July 2026. Of those, 29% of PLHIV have a missed appointment, 61% are defaulters, 7% have an incorrect prescription end date, and 3% are ARV naïve. The prescription end date will continue to be addressed in TSIS data quality meetings and supportive supervision sessions.

Table 15: Global Fund Treatment, Care and Support Indicators as of July 2026

Indicator	Annual Target	Performance	Achievement
People on ART among all people living with HIV at the end of the reporting period.	73.83%	15,080/28,000	54%
^a % of Children (under 15) on ART among all	84.18%	115/129	89%

children living with HIV at the end of the reporting period.			
^b % of People newly diagnosed with HIV initiated on ART.	97%	479/495	97%
^c % of People living with HIV and on ART with viral load test results.	95%	12,936/14,664	88%

^a The number of all children living with HIV is taken from TSIS.

^b The HIV confirmed date is used as a proxy for newly diagnosed PLHIV for 2026.

^c People living with HIV on ART who are eligible (6 months after ART initiation) for a viral load test.

Table 18 displays the Global Fund Treatment, Care and Support targets for 2026. As of July 2026, on a national level, two targets were met/surpassed: 89% of children living with HIV on ART and 97% of people newly diagnosed with HIV initiated on ART.

Table 16: PLHIV Retained on ARV as of July 2026 Matched Against the Annual Target

Region	Parish	PLHIV Retained on ARV	On ARV 2026 Target	Percentage achieved
SERHA	St. Thomas	314	366	86%
	Kingston and St. Andrew	3,128	3,780	83%
	St. Catherine	1,868	2,236	84%
WRHA	Trelawny	379	407	93%
	Hanover	225	246	91%
	Westmoreland	747	767	97%
	St. James	1,889	2,090	90%
NERHA	Portland	317	432	73%
	St. Mary	489	653	75%
	St. Ann	876	1,188	74%
SRHA	Clarendon	835	979	85%
	Manchester	863	978	88%
	St. Elizabeth	377	383	98%
	Total	12,307	14,505	85%

The total number of patients on ARV across the parishes as of July 2026 was 12,307. As of July 2026, no parish has met the On ARV target. However, it is important to highlight the strides made by Westmoreland (97%) and St. Elizabeth (98%).

Table 17: PLHIV Viral Load Testing of those eligible for a viral load test as of July 2026, matched against the Annual Target

Target: 95%

Region	Parish	Viral Load Testing Achieved	Viral Load Testing Target	Percentage Achieved
SERHA	St. Thomas	299	303	99%
	Kingston and St. Andrew	2,494	3,049	82%
	St. Catherine	1,593	1,807	88%
WRHA	Trelawny	355	362	98%
	Hanover	209	215	97%
	Westmoreland	628	730	86%
	St. James	1,717	1,853	93%
NERHA	Portland	296	309	96%
	St. Mary	448	479	94%
	St. Ann	764	851	90%
SRHA	Clarendon	683	814	84%
	Manchester	708	836	85%
	St. Elizabeth	222	357	62%
	Total	10,416	11,965	87%

As of July 2026, St. Thomas, Trelawny, Hanover and Portland have met/surpassed the viral load testing target of 95%.

Table 18: PLHIV Viral Suppression of those eligible for a viral load test as of July 2026, matched against the Annual Target

Target: 95%

Region	Parish	Viral Suppression Achieved	Viral Suppression Target	Percentage achieved
SERHA	St. Thomas	279	303	92%
	Kingston and St. Andrew	2,273	3,049	75%
	St. Catherine	1,374	1,807	76%
WRHA	Trelawny	332	362	92%
	Hanover	190	215	88%
	Westmoreland	554	730	76%
	St. James	1,547	1,853	83%
NERHA	Portland	268	309	87%
	St. Mary	402	479	84%
	St. Ann	697	851	82%
SRHA	Clarendon	621	814	76%
	Manchester	669	836	80%
	St. Elizabeth	181	357	51%
	Total	9,387	11,965	78%

As of July 2026, no parish has met the target.

Key Insights: STI

STI Reporting Compliance Remains Low

- Overall STI reporting compliance was poor, with only 56 of 110 expected reports submitted (51%).
- 52 (47%) of all expected reports were received on time, reflecting a low submission rate which limits the usefulness of STI surveillance data for timely intervention and response.

Key Concern

- Delayed and incomplete reporting may significantly affect:
 - outbreak detection,
 - treatment monitoring,
 - partner notification,
 - and real-time public health action.

STI Burden Continues to Disproportionately Affect Females

- A total of 2,637 STI cases were reported in July.
- Females accounted for the overwhelming majority:
 - 81% females (2,125 cases)
 - 79% males (512 cases)

Key Implication

- This disparity may reflect:
 - higher healthcare utilization among females,
 - stronger female screening systems,
 - or under-detection and under-reporting among males.

The report highlights the need for:

- stronger male-focused outreach,
- increased male STI testing,
- and improved engagement strategies for men.

Young Adults Carry the Highest STI Burden

Most Affected Age Groups

The highest STI burden was concentrated among:

- 20-29 years: 29%
- 30-39 years: 22%
- 40-49 years: 15%

Together, these age groups accounted for approximately 66% of all reported STI cases.

Adolescents and Children

- Adolescents aged 15-19 years represented:
 - 13% of all STI cases
- Children under 15 years accounted for:

- 2% of total cases
- Most Paediatric cases occurred among females aged 10-14 years.

Older Age Groups

- Older adults represented a notable share of reported STI cases
 - 10% - 50-59 years
 - 6% - 60-69 years
 - 2% - 70 years and older

Key Concern

- STI transmission remains concentrated among sexually active young adults, but the presence of cases among minors requires continued safeguarding, adolescent sexual health interventions, and possible child protection review where indicated. The occurrence of STIs among older adults also highlights the need for age-inclusive prevention, testing, and sexual health interventions.

Genital Discharge Syndrome Dominates STI Diagnoses

- Genital Discharge Syndrome (GDS) accounted for the vast majority of STI cases:
 - 2,241 cases
 - represents 85% of all reported STI diagnoses
- Females represented:
 - 85% of GDS (genital discharge and cervicitis) cases

Breakdown

- Genital discharge (males and females):
 - 81%
- Cervicitis:
 - 4%

Key Insight

- Syndromic management continues to be heavily driven by genital discharge presentations, particularly among females.

Genital Herpes Was the Most Reported GUD Cause

- A total of 82 Genital Ulcer Disease (GUD) cases were reported representing 3% of all STI cases.
- 3% of cases attributed to Genital Herpes
- Less than 1% of cases attributed to Chancroid

Key Observation

- No cases of LGV and Granuloma Inguinale were identified during the reporting period.

Late Latent Syphilis Was the Predominant Syphilis Stage

- A total of 144 syphilis cases (5% of all STI cases) were reported.
- Females accounted for more cases overall:
 - 44% - 64 females
 - 56% - 80 males

Stage Distribution

- Late latent syphilis: 3% of cases
- Early latent syphilis: 2%
- Secondary syphilis: Less than 1%
- Primary syphilis: Less than 1%
- Tertiary syphilis: Less than 1%

Positive Findings

- No congenital syphilis cases were reported.

Key Implication

- The predominance of latent syphilis suggests:
 - delayed diagnosis,
 - missed opportunities for early detection,
 - and possible ongoing silent transmission.

Pelvic Inflammatory Disease Was the Most Common Other STI Condition

- 187 individuals were diagnosed and treated for various sexually transmitted infections (STIs) and related conditions (7% of all reported STI cases)

Among other STI-related diagnoses:

- Pelvic Inflammatory Disease (PID) was the most common:
 - 2% of cases
- Genital warts represented:
 - 1% of cases
- Human T-lymphotropic Virus (HTLV) represented:
 - 1% of cases
- Epidemiological treatment for STI exposure was also common.
 - 2% of clients treated for gonorrhoea or chlamydia exposure
 - 1% treated for syphilis exposure

Key Insight

- Female reproductive health complications remain a major component of STI service.

HIV Testing Coverage in STI Clinics Was Moderate

- Total STI clinic attendees for July:
 - 2,344
- HIV testing coverage:
 - 53.6%

Sex Differences

- Males had substantially higher HIV testing uptake despite lower clinic attendance:
 - 63.6% males tested
 - 50.1% females tested

HIV Positivity Yield

- Overall HIV positivity:
 - 2.5% (31 cases)
- Female positivity:
 - 1.3% (12 cases)
- Male positivity:
 - 6.3% (19 cases)

Key Insight

- HIV positivity among STI clinic attendees remains relatively low despite the high-risk nature of STI populations.

Syphilis Testing Coverage Was Moderate

- Syphilis testing coverage reached more than half of clinic attendees for July:
 - 53.6%

Sex Differences

- Male testing uptake: 63.4%
- Female testing uptake: 51.1%

Syphilis Positivity

- Overall positivity:
 - 4.3% (54 cases)
- Male positivity:
 - 11.7% (35 cases)
- Female positivity:
 - 2.0% (19 cases)

Key Concern

- Males demonstrated significantly higher syphilis positivity rates, suggesting:
 - undetected transmission networks,
 - delayed testing,
 - or higher-risk sexual behaviours among male attendees.

Major Programme Gaps

Reporting and Surveillance

- Persistently low reporting compliance, particularly in the South East Region, continues to affect the completeness, timeliness, and overall quality of STI surveillance data.

Male Engagement

- Males remain underrepresented among reported STI cases despite demonstrating higher positivity yields for HIV and syphilis.

Testing Coverage

- HIV and syphilis testing coverage among STI clinic attendees remains below optimal levels, indicating missed opportunities for early diagnosis and linkage to care.

Late Syphilis Detection

- High proportion of latent syphilis suggests delayed diagnosis and missed early intervention opportunities.

Priority Recommendations

1. Strengthen STI reporting compliance through routine internal data quality and reporting checks, with targeted follow-up in regions that consistently submit reports late.
2. Expand male-focused STI outreach, testing, and prevention services.
3. Increase HIV and syphilis testing coverage among STI clinic attendees to maximize opportunities for early diagnosis and timely linkage to care.
4. Strengthen early syphilis screening and active case finding to reduce late latent presentations and facilitate earlier treatment.
5. Enhance targeted sexual health education and prevention programmes for young adults, who continue to carry the highest burden of reported STIs.
6. Enhance integrated reproductive health services for women, especially around Pelvic Inflammatory Disease management.

STI Cases Reporting Compliance

In July 2026, a total of 110 reports were expected across the four regions; however, only 56 were received, resulting in an overall reporting rate of 51%. A total of 52 submitted reports were received on time, resulting in an on-time submission rate of 47%. Additional updates are often submitted later in the month to improve reporting compliance; however, despite early data entry access, timely reporting compliance remains low.

The Western Region demonstrated the highest performance by submitting 19 of 19 expected reports (100%), with 100% submissions made on time (Table 19), maintaining their streak of perfect submission from the previous quarter.

The North East Region followed, submitting 11 of 13 reports (85%), of which 10 (77%) were on time. The Southern Region submitted 22 of its 25 expected reports (88%), with 19 (76%) timely submissions received within the required timeframe. The South East Region consistently records the lowest performance, submitting 4 of 53 expected reports (8%) for the quarter, with all 4 reports (8%) received on time (Table 19).

While most regions have demonstrated improved reporting compliance, targeted efforts are still needed to strengthen timely reporting within the South East Region. The region has reported staffing challenges and while Contact Investigators (CIs) generally submit reports to their supervisors within the expected timeframe, subsequent data quality checks and validation processes contribute to delays in submitting finalized reports to the DHIS2 database.

Table 19: Reporting Compliance for STI Cases – July 2026

Region	Expected Reports	Actual Reports	Percent Received	Reports On Time	Percent On Time
Western Region	19	19	100%	19	100%
North East Region	13	11	85%	10	77%
Southern Region	25	22	88%	19	76%
South East Region	53	4	8%	4	8%
Total	110	56	51%	52	47%

STI Surveillance

A total of 2,637 sexually transmitted infection (STI) cases were reported for the month of July, with 512 (19%) cases among males and 2,125 (81%) among females (Figure 12). The distinct difference in STI cases by sex underscores the need to strengthen male-focused testing and prevention strategies to support more balanced case detection. However, the notable decrease in STI case reporting observed in July (51%) compared to previous months (85%-89%) should be interpreted cautiously, as it is likely due to delayed reporting by Contact Investigators despite early data entry access.

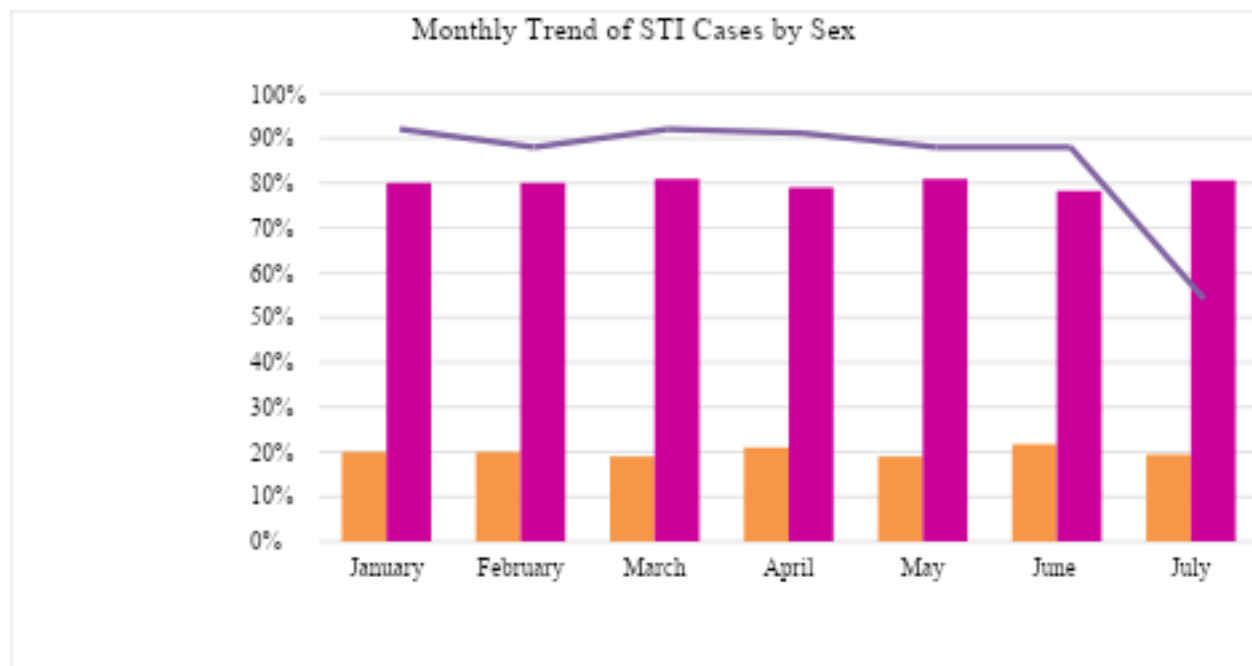


Figure 12: STI Cases by Sex – July 2026

Age-Group Disaggregation

A total of 2,637 sexually transmitted infection (STI) cases were reported for July, with 2,125 (81%) cases among females and 325 (19%) cases among males. The highest burden of STI cases was observed among individuals aged 20 to 29 years, accounting for 776 (29%) of total cases. There were 578 (22%) of the total cases reported within the 30-39 years' age group and 399 (15%) cases recorded in the age band 40-49 years.

STI cases were also significantly observed in older populations, with the 50-59 age group contributing 256 (10%) cases. The 60-69 and 70+ age group contributed 146 cases (6%) and 67 cases (2%), respectively (Figure 13).

Among adolescents aged 15-19 years, 334 (13%) of the total cases were reported while children under 15 years of age accounted for 47 (2%) cases, with the majority (43 of 47) of the cases being females aged 10-14. The 0-9 years' age group accounted for 29 (1%) of the total reported cases for the period (10 males, 19 females). Five unknown cases were reported, representing less than 1% of total STI cases for the reporting period (Figure 13).

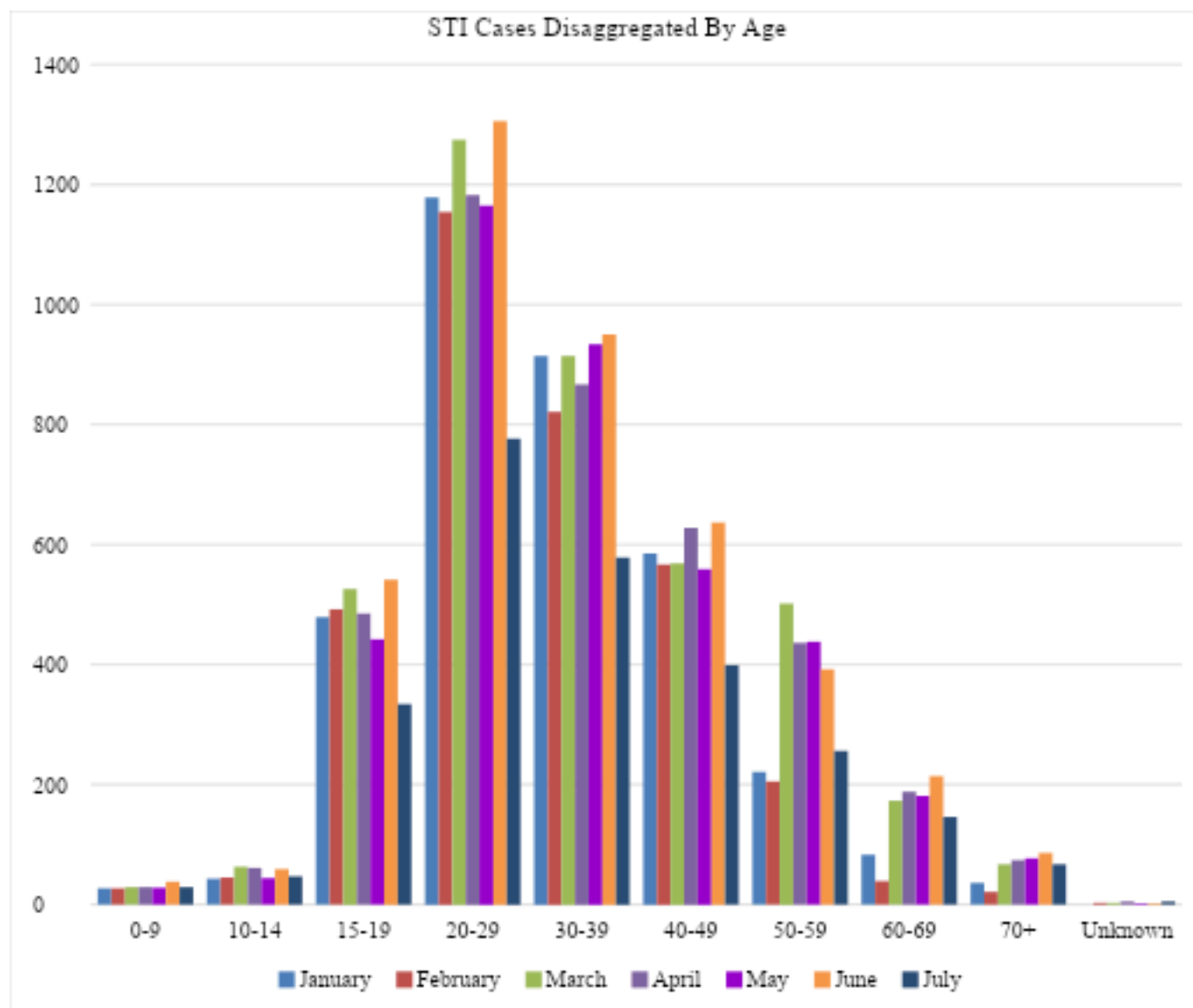


Figure 13: STI Cases Disaggregated by Age –July 2026

Trends in STI Syndromes and Conditions by Month

In July, Genital Discharge Syndrome (GDS) remained the predominant STI syndrome, accounting for 85% of all reported STI cases. Genital Ulcer Disease (GUD) represented 3% of reported cases, while syphilis accounted for 5%, reflecting a slight increase compared with previous months. Other STI and non-STI conditions contributed 7% of the overall STI burden and were primarily driven by epidemiological treatment for chlamydia and syphilis along with conditions such as pelvic inflammatory disease (PID) and genital warts (Figure 14).

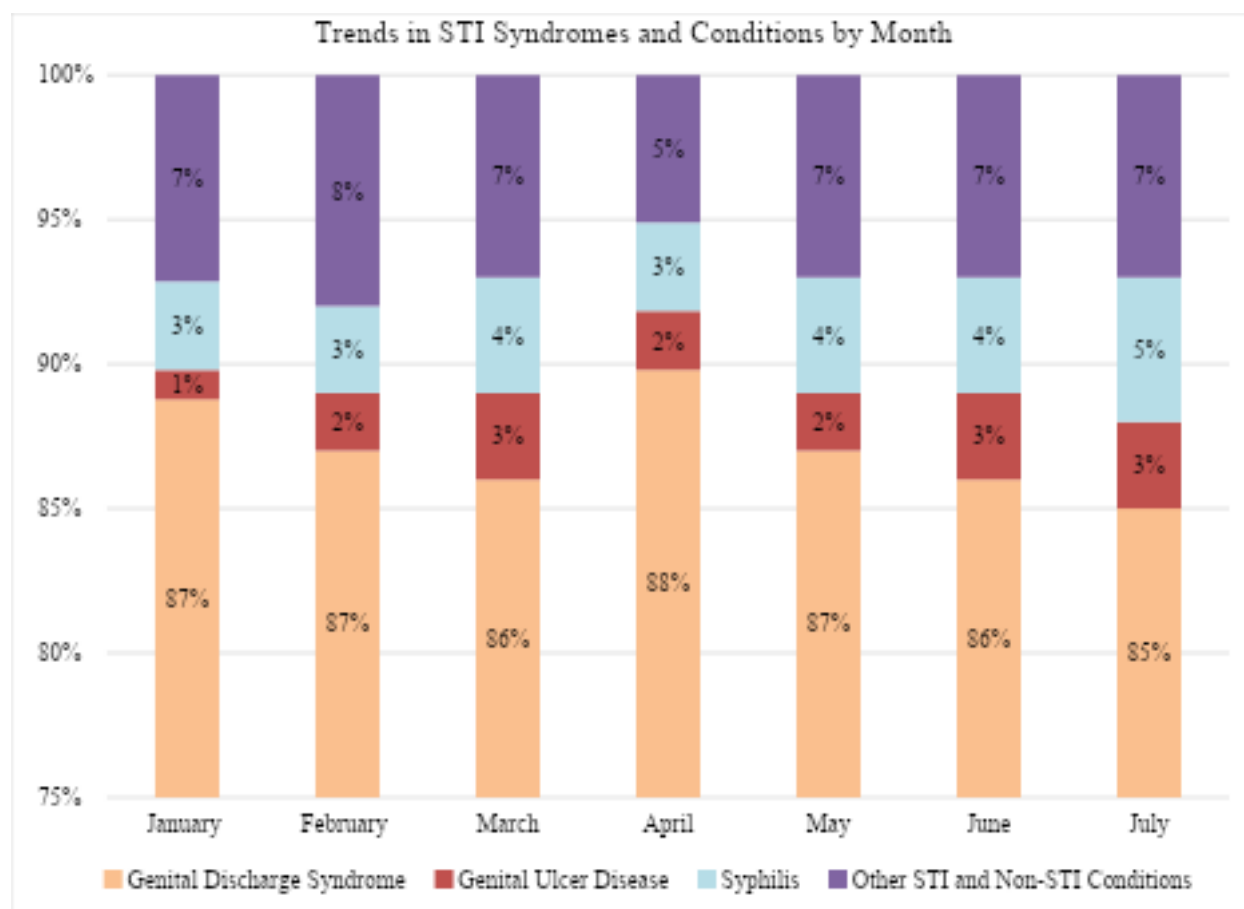


Figure 14: Trends in STI Syndromes and Conditions by Month

STI Breakdown

A. Genital Discharge Syndrome

A total of 2,241 cases of Genital Discharge Syndrome were recorded in July representing 85% of total STI cases reported. A significantly higher burden was observed among females with 1,897 (85%) cases compared to 344 (15%) cases in males. Among these cases, genital discharge was the predominant presentation, affecting 2,147 individuals, of which 344 are males and 1,803 are females, representing 81% of total STI cases reported. Cervicitis, a condition specific to females, accounted for 94 (4%) cases (Figure 15).

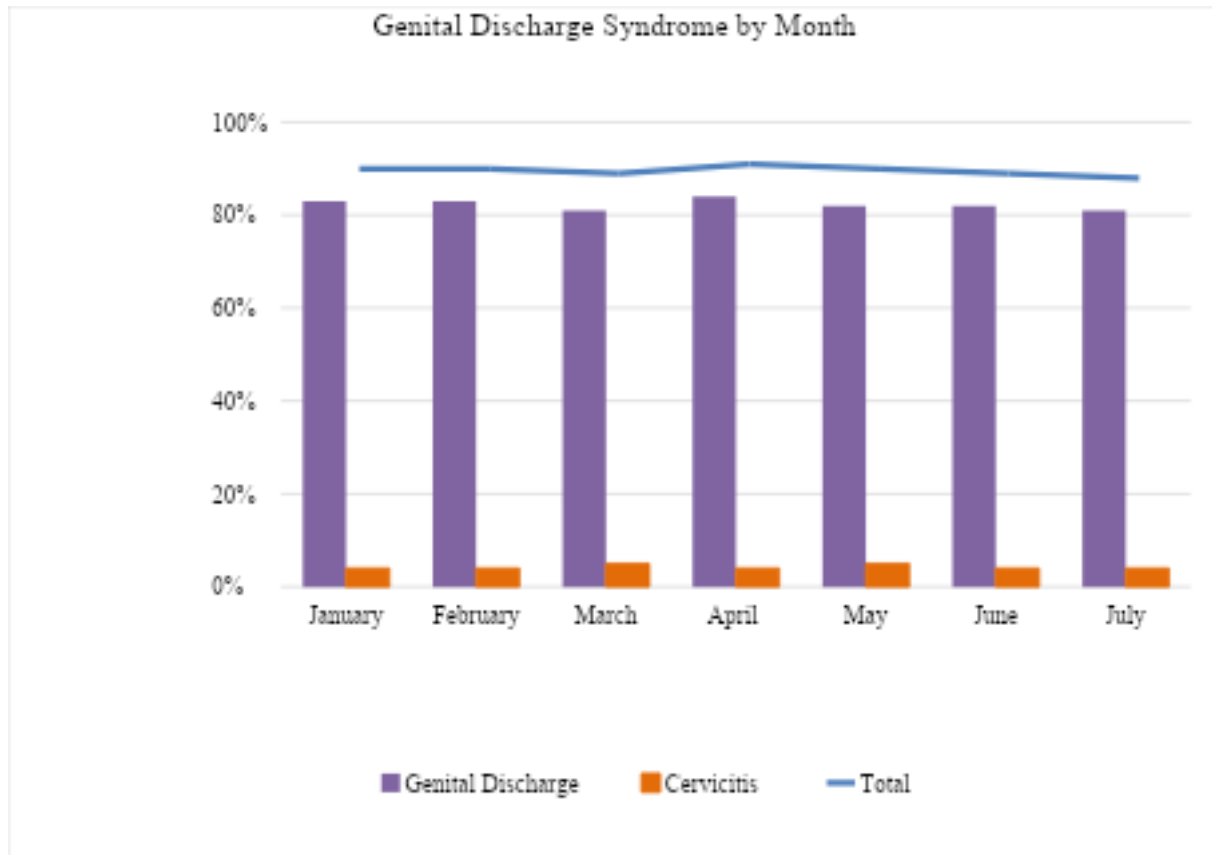


Figure 15: Distribution of Genital Discharge and Cervicitis Cases – July 2026

B. Genital Ulcer Disease (GUD)

A total of 82 Genital Ulcer Disease (GUD) cases were reported during the reporting period, representing 3% of all STI cases. Genital herpes was the predominant aetiology, accounting for 81 cases (3% of total STI cases). Female reported cases exceeded those among males, with females accounting for 78% (63 cases) and males for 22% (18 cases). One male case of Chancroid was also reported, representing less than 1% of total STI cases. Granuloma Inguinale and Lymphogranuloma Venereum (LGV) were not reported during the period (Figure 16).

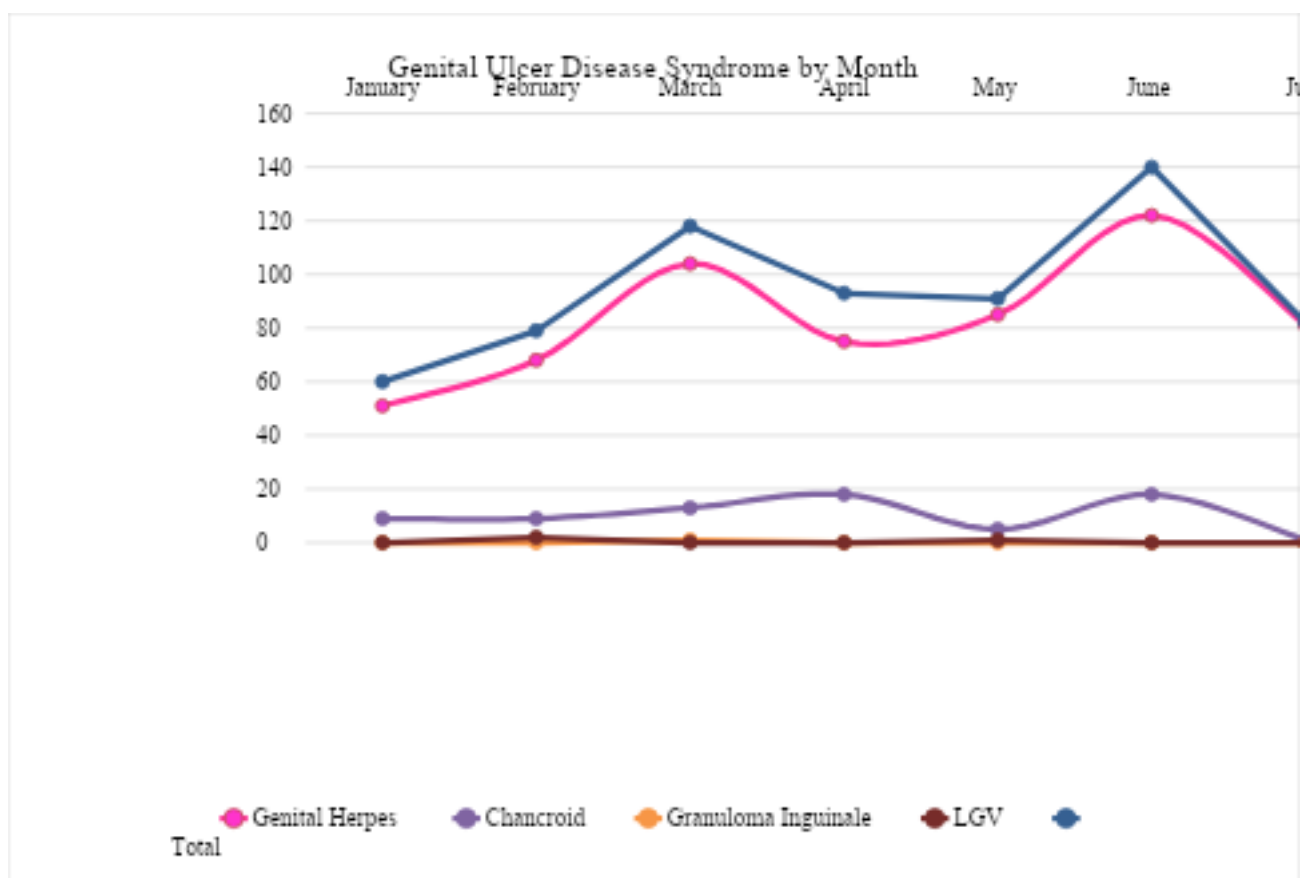


Figure 16: Distribution of Genital Ulcer Disease Syndrome Cases – July 2026

C. Syphilis Cases

A total of 144 syphilis cases were reported in July representing 5% of all STI cases reported, with a slightly higher burden observed among females (64 cases, 44%) compared to males (80 cases, 56%). Late latent syphilis was the predominant stage, accounting for 3% of all reported cases (71), comprising 37 males and 34 females.

Early latent syphilis (A3) accounted for 2% (43 cases) of all reported syphilis cases, comprising 19 females and 24 males. Secondary syphilis (A2) accounted for less than 1% of reported syphilis cases, with 13 cases (5 females and 8 males). Additionally, 16 primary syphilis (A1) cases were recorded, comprising 5 females and 11 males, representing less than 1% of total reported syphilis cases for July. One female case of tertiary syphilis was also reported, accounting for less than 1% of all syphilis cases during the reporting period (Figure 17).

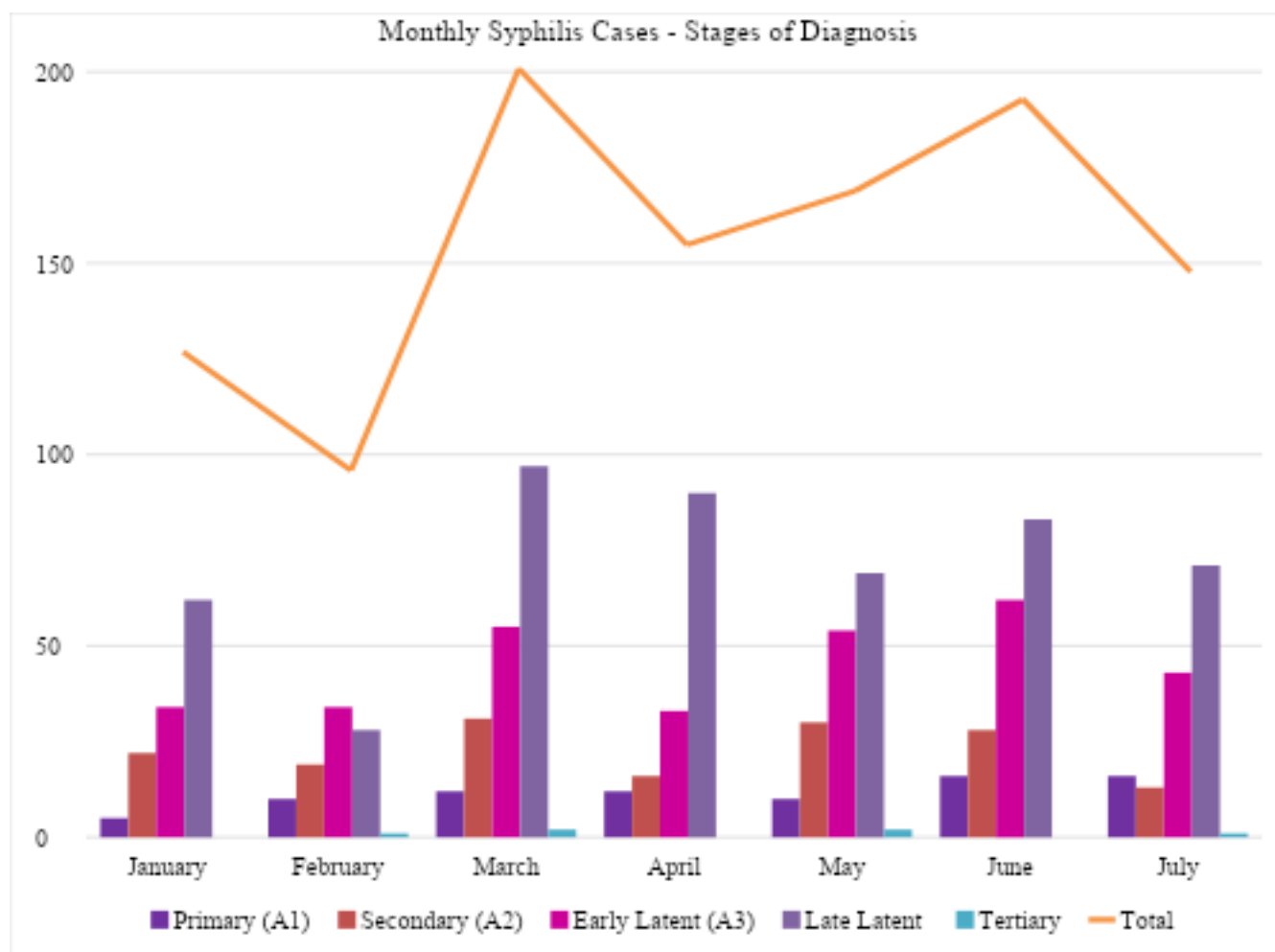


Figure 17: Distribution of Syphilis Cases – July 2026

D. Other STI and Non-STI Conditions

A total of 187 individuals were diagnosed and treated for various sexually transmitted infections (STIs) and related conditions during June, representing 7% of all reported STI cases. Females accounted for 62% (116 cases) of these diagnoses, while males represented 38% (71 cases).

Epidemiological treatment was provided to 76 individuals. Of these, 23 individuals (1% of total STI cases) received treatment following exposure to syphilis, while 53 individuals (2%) were treated following exposure to gonorrhoea or chlamydia (Figure 18).

Pelvic Inflammatory Disease (PID) was the most frequently recorded condition, accounting for 2% (59 cases) of all reported STI cases. Epididymo-orchitis accounted for 5 cases, representing less than 1% of total STI cases.

Genital warts accounted for 1% (17 cases) of reported STI cases, affecting 12 females and 5 males. Ophthalmia neonatorum represented less than 1% (13 cases) of total STI cases, with 4 female and 9

male cases reported. One male case of hepatitis B/C was recorded, representing less than 1% of total STI cases. Additionally, 16 cases of Human T-lymphotropic Virus (HTLV) I/II were reported (10 males and 6 females), accounting for 1% of all reported STI cases. The number of HTLV I/II cases showed an increasing trend over the course of the year (Figure 18).

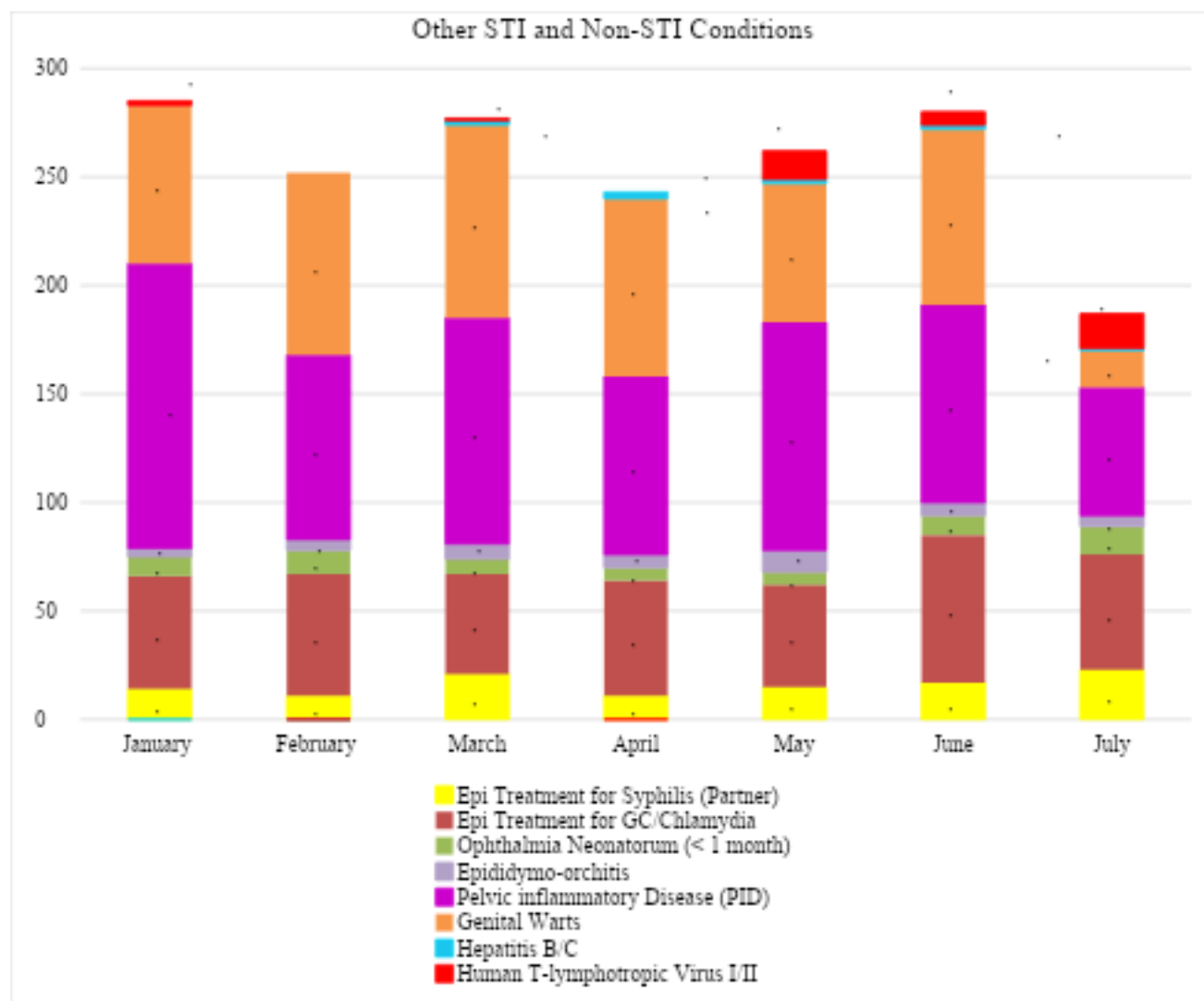


Figure 18: Distribution of Other STI and Non-STI Conditions – July 2026

STI Clinic Testing and Surveillance

During the reporting period July, a total of 2,344 individuals attended STI clinics, comprising 473 males (20.2%) and 1,871 females (79.8%). (Table 20).

HIV Testing Coverage and Yield

HIV testing was conducted for 1,256 attendees, reflecting an overall testing coverage of 53.6%. Males demonstrated a notably higher HIV testing rate, with 301 out of 473 males tested (63.6%) compared to 955 out of 1,871 females (50.1%).

Despite the higher testing coverage among males, the HIV positivity yield remained relatively low at 6.3% (19 positive cases). However, females produced a lower positivity rate of 1.3% (12 positive cases) despite having more persons tested. As a result, the overall positivity rate was 2.5% (31 cases). (Table 20).

Syphilis Testing Coverage and Yield

Syphilis testing coverage reached 53.6%, with 1,256 individuals tested among the total clinic attendees during the reporting period. Testing uptake was higher among males, with 300 of 473 (63.4%) tested, compared to 956 of 1,871 females (51.1%).

There was a reported syphilis positivity yield of 4.3%, representing a total of 54 cases, 35 among males (11.7%) and 19 cases among females (2.0%).

During the reporting period, it was observed that female attendees accounted for the majority of STI clinic visits; however, males were more likely to be tested for both HIV and syphilis and had higher positivity yields for both infections, highlighting the importance of strengthening testing coverage among females while maintaining targeted case-finding among males (Table 20).

Table 20: STI Clinic Testing and Surveillance – July 2026

Sex	STI Clinic Attendees	Tested for HIV (%)	Positivity Yield (HIV)	YTD Positivity Yield (HIV)	Tested for Syphilis (%)	Positivity Yield (Syphilis)	YTD Positivity Yield (Syphilis)
Males	473 (20.2%)	301 (63.6%)	19 (6.3%)	77/3,525 (2.2%)	300 (63.4%)	35 (11.7%)	198/3,540 (5.6%)
Females	1,871 (79.8%)	955 (51.0%)	12 (1.3%)	70/8,749 (0.8%)	956 (51.1%)	19 (2.0%)	204/8,641 (2.4%)
Total	2,344	1,256 (53.6%)	31 (2.5%)	147/12,274 (1.2%)	1,256 (53.6%)	54 (4.3%)	402/12,181 (3.3%)

TUBERCULOSIS

TB Surveillance

Surveillance Data (YTD) – Latest MOHW Epi Weekly Report (Monthly)

Table 21: Number of TB Cases Confirmed – July 2026 vs July 2025

As of July 2026, the surveillance data shows:

Event	YTD 2026	YTD 2025
Tuberculosis	43	39

As of July 2026, the TB Database Register contains approximately 155 clients, the majority of whom were confirmed in 2025.

Table 22: Confirmed TB Cases entered in TB Database by Year

As of July 2026, the database register shows:

Year	No. of Cases
2022	1
2023	2
2024	5
2025	90
2026	57

HEALTH INFORMATION SYSTEMS

1. Treatment Site Information System 2 (TSIS2)

a. Support to Site Users (ongoing)

- Responded to user queries related to TSIS.
- Managed user access for:
 - TSIS Data Entry
 - TSIS Reports
- Created new users and modified existing user accounts.
- Installed and applied client-side certificates on user computers.

b. Support for Data Cleaning Activities (July)

The following data cleaning activities were completed:

- Transfers: 23
- Mergers: 14
- Client deletions: 23

c. Ad-hoc Reporting

- Demographic data request from WRHA TCSOs actioned
- Annual review of data request from SI M&E Officer actioned

2. District Health Information Software 2 (DHIS2)

a. EMTCT

- Weekly database backups were completed.
- Maintained the following registers:
 - HIV-Exposed Infants (HEIs)
 - Maternal HIV
 - Maternal Syphilis

b. STI and CI Monthly Reports

- Provided ongoing support to staff.
- Adjusted STI and CI reports incorrect calculations; final adjustments will be completed in August
- Configured database to allow users to access the STI/PMTCT dashboard via their usernames and passwords

c. Tuberculosis (TB) Database

- Provided direct support to regional data entry staff via phone.
- TB certificates installed upon request.
- Provided ongoing monthly extracts of TB entries to a designated TB focal point.
- Troubleshooting support was provided using remote support/control software.
- Assisted users with navigating the TB database, including key data entry fields and the reports section.
- Reset user passwords as required.

d. Prevention Database

- Provided ongoing end-user support.
- Compiled data for:
 - Global Fund Payment for Result (PFR)
 - SI Report
 - Grants Progress Report
 - MOF Report
- De-duplication report sent to CSOs and RHAs.
- Validation checks completed for monthly reports

3. Other Activities

- LMS support continues with consultant
- PFR data validation exercise to use for national reporting.
- STI/PMTCT dashboard development completed sensitization sessions to be held in August.
- Support being provided for TSIS-DHIS2 Interoperation consultancy.
- Add Client Retention in Care and on ARV Section in TSIS2 database
- Added HIV Drug Resistance Module in TSIS2 - Awaiting final demonstration/review
- Create TSIS2 duplicates listing

SI ACTIONS FROM TECHNICAL WORKING GROUP MEETINGS

Table 23: SI Actions from Technical Working Group Meetings

Meetings	Actions	Responsible Officer	Status
TSIS Data Quality Meeting	List of archived first ARV dates to be sent out to determine if the first ARV dates should be re-entered or remain archived in TSIS.	Strategic Information Component	Ongoing
	Review Other Labs to ensure they capture all the relevant labs for HIV treatment.	Treatment, Care and Support Component	Ongoing
	Review the client dashboard and report dashboard.	Strategic Information Component	Ongoing
	Include a checkbox to acknowledge that the request was seen, or a receipt in TRT.	Strategic Information Component	Ongoing
	Introduce a standard day/job aide for when certain requests are actioned from the TRT.	Treatment, Care and Support Component	Ongoing
	Add a sheet with the guidelines in the TRT.	Strategic Information Component	Ongoing
	Data Pack to be sent out by the end of September for action, next meeting Nov 19-20.	Strategic Information Component	Ongoing
	Revise the case archiving protocol regarding CD4 and the transfer and de-duplication protocol regarding duration of time or visits patients should be attending transfer/new site.	Strategic Information Component and Treatment, Care and Support Component	Ongoing
	Conduct a refresher for the retention linkage protocol	Strategic Information Component and	Ongoing

Meetings	Actions	Responsible Officer	Status
	EMR to be updated with the 2025 version of the outpatient intake tool	Treatment, Care and Support Component	Ongoing
	Discussions to be held around the SLA targets	Strategic Information Component	Ongoing
	TSIS Psychosocial training to be held with the RHAs psychosocial team.	Strategic Information Component	August 2026
PMTCT Data Review Meeting	Database updates will be implemented to introduce a data completeness cut-off date, ensuring that reports submitted on time are not affected when subsequently re-accessed for revisions or updates.	Strategic Information Component	Ongoing
M&E TWG Meeting	Discuss internally to incorporate qualitative research in the operational research initiative and liaise with Dr, Jarrett to provide technical support.	Strategic Information Component	Ongoing
	Proposed research agenda and presentations to be shared with TWG.	Strategic Information Component	Ongoing
Data-O-Rama Planning Meeting	Letters to be sent out to the RHAs and Non-RHAs and to the facilitators regarding their presentations.	Nardia Chulan	Ongoing
TSIS-DHIS2 Interoperation Consultation	Update the TSIS variable definitions	Strategic Information Component	Ongoing
Operational Research Workshop Consultation	Consultant engaged, operational research agenda, ethics proposal and workshop dates to be finalized	Operational Research Consultant, Strategic Information Component	Ongoing

Meetings	Actions	Responsible Officer	Status
SPSS Workshop Consultation	Consultant engaged, training materials and course outline and workshop timeline to be finalized.	SPSS Consultant, Strategic Information Component	Ongoing
WRHA Supportive Supervision	Conduct monthly data monitoring activities to identify gaps and assist sites in developing remedial actions.	SI, Focal Point	Ongoing
NERHA Supportive Supervision	Conduct an Excel Training Session.	Strategic Information Component	November 2026
SRHA Supportive Supervision	Monthly gap analysis to identify gaps in data for Paediatric sites with frequent loss to follow-ups.	Strategic Information Component, Focal Point	Ongoing
University Hospital Paediatrics (UHWI Paeds)	Conduct routine data cleaning and monitoring activities and provide feedback on all action items.	SI - Data Entry Clerk	Ongoing
Department of Correctional Services (DCS)	Establish a formal transfer protocol for patients transitioning between correctional facilities to ensure continuity of care.	DCS & SI - Data Entry Clerk	Ongoing

Major Initiatives/ Activities	Key Outputs	Performance Measures/Indicators	Targets	
			July-August	Status
Quarterly HIV monitoring cascade report	Quarterly HIV monitoring cascade report	4 Quarterly HIV monitoring cascade reports	Approved Quarterly HIV monitoring cascade report (Apr-Jun) submitted	In Progress
Quarterly STI Database Report	Quarterly STI Database Report	4 Quarterly STI Database reports	Approved Bi-Annual STI Database report submitted	Completed
Quarterly PMTCT Database Report	Quarterly PMTCT Database Report	4 Quarterly PMTCT database reports	Approved Bi-Annual PMTCT database report submitted	Completed
Quarterly HIV Prevention Database report	Quarterly HIV Prevention Database report	4 Quarterly HIV Prevention Database reports	Approved Bi-Annual HIV Prevention Database report submitted	Completed
Quarterly TB Database Report	Quarterly TB Database Report	4 Quarterly TB Database reports	Approved Bi-Annual TB Database report submitted	Completed
Assessment of HIV Data Quality	HIV Data Quality Audit reports	HIV DQA was conducted with at least 60% of corrective actions implemented	2026 HIV Treatment Data Quality Audits Reports submitted to D, HPPB (SERHA, NERHA, SRHA)	In Progress
PMTCT Data Review Meeting	PMTCT Data Review Meeting Reports with corrective action plans	4 PMTCT data review meeting reports	Approved PMTCT Data Review report submitted	Upcoming (August 18, 2026)
Development of TB Data	TB Audit tool developed	TB data quality audit tool pre-tested and finalized	Final TB Audit tool submitted	Completed

Major Initiatives/ Activities	Key Outputs	Performance Measures/Indicators	Targets	
			July-August	Status
Quality Audit tool				
Development of STI Data Quality Audit tool	STI Audit tool developed	STI data quality audit tool pre-tested and finalized	Final STI Audit tool submitted	Completed
Supportive Supervision Visits	2 Supportive Supervision Visit reports	Supportive Supervision Visits were conducted as per schedule, and 80% of data quality gaps were addressed	2 Approved Supportive Supervision visit reports submitted	Completed
M&E Technical Working Group Meeting	M&E Technical Working Group Meeting	2 action sheet updates from M&E TWG meetings	M&E meeting held with key stakeholders	Upcoming (September 30, 2026)
TSIS Quarterly Data Review Meeting	TSIS Quarterly Data Review Meeting	80% of actions completed on plans	TSIS Quarterly Data Review Meeting and approved report	In Progress (Meeting was held August 3-5, 2026)
Quarterly HIV/STI/TB Database Bulletin	Quarterly HIV/STI/TB Database Bulletin	3 HIV/STI/TB Database Bulletins	Quarterly HIV/STI/TB Database Bulletin approved	In Progress
Assessment of Prevention Data Quality Audit	Approved Prevention Data Quality Audit reports	Prevention DQA conducted with at least 60% of corrective actions implemented	Prevention DQA report & action plan - WRHA	In Progress (awaiting memo approval)
Strategic Information Training Report	Strategic Information Training Report	Number of HCW trained SI management	SPSS Workshop (consultancy)	In Progress (consultant engaged)

Major Initiatives/ Activities	Key Outputs	Performance Measures/Indicators	Targets	
			July-August	Status
HIV/STI/TB Data Triangulation Meeting	HIV/STI/TB Data Triangulation Meeting	80% of data triangulated	HIV/STI/TB Data Triangulation Meeting and approved report.	In Progress (Awaiting feedback from PPD)
Updated/Integrated Treatment and Prevention Databases	Final report on the updating and integration of the treatment and prevention databases	Updated/Integrated Treatment and Prevention Databases operationalized	Consultant recruited to revise and integrate both databases	Completed
Data Repository Monitoring	Data Repository Updates	Number of updates on published and unpublished studies/reports	Data Repository Updates Report	In Progress
Conduct research on HIV/STI/TB	Operational Research Abstracts	2 Operational Research abstracts approved	Operational Research Workshop	In Progress (August 2026)

The table below provides an update of the activities assigned to the SI Component in the medium and long term

Activity	Update
Replacement of damaged equipment in affected parishes	Procurement has commenced to replace these equipment
Update HST Programme audits tools to include a disaster risk management (DRM) component	This will be done in Q4 when the audit tools are being revised
Create DRM job aides	To be done in Q2
Integrate a DRM component in HST Programmes reviews and meetings	To be done starting in Q2